

The Dementia Caregiver's First Year

*A Practical Guide to Diagnosis, Safety, Hard Days, and
Protecting Yourself*

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A Plan Before the Next Hard Day

Mama was humming over a cup of tea. The spoon lay on the saucer, and her thumb kept time against the handle. I knew the hymn. She'd played it at church for years, back when she could tell you which soprano had come in early and exactly how early she'd come in.

By then, Dot couldn't give me a full sentence. The tune came through anyway.

I sat down instead of picking up the cup.

That moment belongs later in our story, not to the day of her diagnosis. Dot was a person I loved, with a sharp ear and opinions about nearly everything. A care plan was supposed to serve her.

The tea had gone pale. I had been standing with one hand ready to take away the saucer, already thinking about the next thing waiting for me. Sitting put my ears closer to the little tune. She reached the part I remembered, then went back over it. I didn't need to find a lesson in every note.

There was still work waiting when I got up. Sitting hadn't canceled it. But I wanted arrangements that left room for being her daughter, not just the person who arrived carrying the next job. That small distinction mattered to me at the table.

You may be reading with an appointment card beside you. Perhaps your parent has a diagnosis. Perhaps you're still waiting for an explanation and don't know what to call the changes you've seen. You can want answers and dread getting them at the same time.

You don't have to settle that feeling before doing the next useful thing.

The job nobody handed over

There may be no clear moment when you become the lead caregiver. A call comes to you because you answered the last one. You agree to make an appointment. Someone asks you to keep the details, since you already have them. Before long, the word “we” in a family conversation seems to mean you.

Consider how a small favor can grow. You offer to find an office’s opening hours. Now you have the number, so someone asks you to make the call. When the office hasn’t replied, everyone asks you for news. The original favor took a minute; the waiting and explaining have no end marked on them. Nobody has asked whether you can keep doing that part.

Being nearby can put you in that position. So can having a flexible lunch break, or simply being willing to answer. None of those things tells the family how much time you actually have. A lunch break is not an empty afternoon.

I would separate the favor you accepted from the continuing job people have attached to it. You can agree to get an answer this time and still say that someone else must handle the next request. That is a change in the work, not a withdrawal of affection.

If that sounds familiar, I have an objection to the advice that you should take everything one day at a time. Some days, yes. But an appointment needs making before the morning you need to attend it. Help needs asking for while you still have enough attention to explain the job.

I spent years dispatching trucks for a freight company. A driver with a flat needed to know who was coming and what happened to the delivery. When I began helping care for Mama, I wanted that same certainty.

I couldn’t have it. I could give a job a name, though, and stop relying on everybody remembering the same conversation.

I'll tell you what helped: making a few arrangements while the day was still ordinary. That is what I mean by building systems. Not buying an app. Not making your kitchen look like an office. An arrangement that somebody else can understand without you standing beside them is enough.

One arrangement might concern family updates. Suppose you've answered the same question in three separate messages and then been asked why you didn't tell a fourth relative. You could agree that ordinary updates will go in one family message thread after the person handling a task reports back. "The office called; the appointment is confirmed for Tuesday" belongs there once. Urgent concerns still need direct contact with someone who can respond; a message thread isn't emergency help.

Another arrangement might make an offer usable. "I can help on Friday" leaves you guessing whether that means a whole day or twenty minutes. "I can handle the return call between nine and eleven, but I'm at work after that" gives you something definite. You might not like the limit. At least you won't build Friday around help that disappears at lunchtime. No one should have to disclose their whole private life to explain that they aren't available.

A third might settle what happens when an answer hasn't arrived. Someone has called, left a message, and heard nothing. That isn't the same as forgetting the job. Agreeing that they will report the unanswered call on Wednesday lets you distinguish waiting from work nobody accepted. It also gives you a chance to decide who follows up, rather than silently taking it back yourself.

These are examples of agreements, not a new filing system to maintain. Choose one that removes an actual piece of guessing from your day.

I want the first year to be about getting those arrangements in place before you're trying to invent them during a hard afternoon.

That doesn't mean a plan can prevent every crisis. It means you shouldn't also have to remember who agreed to return a phone call.

You may already be well past the diagnosis visit. Start where you are.

An owner, a drop date, and no guilt in the driver's seat

My first rule is that every task gets an owner and a drop date. Dispatch language. A drop date is the agreed day the job will be done, or the day its owner reports back if it can't be done yet.

"We should ask about that" is a wish. "I'll call the office Tuesday and tell you what they say" is a job somebody has accepted. If the office can't answer, the next step still needs a person attached to it.

The owner doesn't have to be you. Nor does the person doing a job have to do it my way. I care whether the call gets made, not whether somebody uses my favorite pen.

Being the contact person is different from being the person who does everything. If your family has handed you both roles without discussing either, you are allowed to ask for a different arrangement.

My second rule is that guilt is not a planning tool.

It can tell you that you should have noticed earlier. It will happily occupy the whole evening without making a single phone call.

That's guilt. Set it down.

I don't mean you'll stop feeling it because I said so. I mean it doesn't get to decide who is available on Thursday. Your health and the actual help available belong in that decision. A plan that requires you to be available without interruption isn't one I'm going to recommend.

You can love your parent and say that an arrangement isn't workable. You can be disappointed with the options and still choose

the best one you can actually carry out. I won't ask you to feel cheerful about it.

Read for the week you're in

The chapter numbers below begin at 10. Use the labels as printed; Chapter 15a is the daily-care chapter, not a page you've missed.

This week, read Chapters 10, 11, 13 and 14. This month, handle Chapters 15a, 16, 17 and 18. Return to Chapters 12, 15 and 19 as communication, behavior and care needs change.

The first group starts with understanding the diagnosis and preparing for doctor visits, then turns to home safety and driving. The next group makes room for daily care, early legal and financial questions, paying for care, and your own health. The chapters you'll revisit concern communication, difficult behaviors, and considering memory care.

That's a reading route, not a requirement to wait. An immediate health or safety concern takes precedence over finishing a chapter. Seek appropriate professional or emergency help rather than reading on to see whether your situation appears in an example.

If a conversation is going badly today, go to Chapter 12 today. If you need to consider memory care now, Chapter 19 is there now. Reading ahead isn't a prediction about your parent. You can gather information without making a decision that afternoon.

You also don't owe this book an uninterrupted weekend. I would rather you use one page than finish the book and feel you have acquired another set of obligations.

An interruption doesn't mean you have to begin again at the first page. Suppose work called you away for several days, and you come back to the book with a different appointment date and an unanswered question. The old bookmark tells you where you stopped reading. It doesn't tell you what matters now.

Take the question that's still live and go to its chapter. If a clinician or another professional has given you instructions, follow those rather than substituting the book's reading order. Then return to the week or month group for anything still unresolved. Reading a chapter and settling the matter it raises are different things. You may need another conversation before an answer is useful.

You can also leave a chapter alone when it isn't needed today. Put a brief note beside its title about what would bring you back: a question you couldn't answer, for instance, or a decision your family has begun discussing. That's a place to resume, not a prediction of decline.

The route can be revised without declaring the whole plan a failure. Keep an arrangement that still works. Change the one that no longer fits the help available. A missed reading week doesn't erase a call somebody already made.

What these pages can do

This is a book about the work around caring for a parent with dementia. It offers ways to prepare a question, share a task, and notice what you need help answering. It isn't medical, legal or financial advice.

For questions about symptoms or treatment, contact your parent's treating clinician: an answer needs their health history, not a resemblance to a story here. Don't start, stop or change medicines because of something you read in this book. Questions about legal authority or documents belong with a licensed elder law attorney; rules differ by state, so ask how yours applies. For a financial decision specific to your family, get qualified advice before acting.

Where a chapter discusses a health fact or a coverage rule, it names the source so you can check the current information. The National Institute on Aging, Medicare.gov and the Centers for

Medicare & Medicaid Services are among those sources. Local services and program rules can change. A paragraph printed here can't confirm what is available to your parent today.

For general downsizing and moving logistics, the companion book *Downsizing a Parent* covers that ground.

Margaret "Maggie" Holloway is a pen name, and the personal stories here are composites written to illustrate real situations, not accounts of identifiable families. I claim no professional credential. The stories aren't evidence that a choice will suit your parent.

For tonight, open Chapter 10. Read until you find one question you need answered, then write it down in your own words. Leave room underneath for the answer.

Chapter 10: Understanding the Diagnosis

The appointment is over, but the daughter hasn't put her pen away. In this imagined diagnosis-day conversation, her father sits beside her, holding the folded sheet the office gave him. She has written "dementia" near the top of her notebook. Below it she has written "Alzheimer's?" with a question mark.

He looks at the page. "Which one did they say I have?"

She isn't sure whether those words mean the same thing. She heard the explanation. She also heard the word she had been dreading, and the rest became harder to hold on to.

The useful next move is to ask the office to clarify what the clinician concluded. Not to choose a diagnosis from a search result while sitting in the parking lot.

If you're in that parking lot, leave your notebook open. This week's job is to understand what has been diagnosed, what remains uncertain, and what the clinician wants to happen next. You don't have to turn yourself into a person who can read a brain scan.

What the word dementia does and doesn't tell you

The National Institute on Aging, part of the National Institutes of Health, describes dementia as a decline in thinking abilities serious enough to interfere with everyday life. Remembering is part of it. So are reasoning and understanding. Its [dementia overview](#) makes another point plainly: dementia is not a normal part of aging.

That matters when someone dismisses a concern with “Well, he’s getting older.” Growing older doesn’t by itself explain a loss of ability that needs medical attention. On the other hand, one forgotten name doesn’t establish dementia either. You need an assessment, not a family vote between “nothing’s wrong” and the worst thing anyone can imagine.

Dementia describes a set of difficulties. It doesn’t name one disease. Alzheimer’s disease is one cause; other causes can produce overlapping difficulties. Think of “dementia” as the broad description and the type as information about what is causing it. You can ask for both in ordinary language.

According to NIA, symptoms can involve trouble expressing thoughts, changes in judgment or personality, and problems with movement, as well as memory loss. A parent who remembers an old address may still have difficulties elsewhere, and trouble finding words needs evaluation even if the family hasn’t noticed much forgetting.

Your parent’s preferences still belong in a conversation about care. I would keep the person in the conversation rather than let the paperwork become the only voice in the room.

If you haven’t received a diagnosis, contact the primary care office about the changes you’re seeing. Don’t use the descriptions below to decide which disease your parent has. Their purpose is to make the words less strange when a clinician uses them.

The type gives you a better question

NIA’s overview names Alzheimer’s disease, vascular dementia, Lewy body dementia, frontotemporal dementia and mixed dementia among the types. These aren’t five separate boxes into which a family can sort symptoms at home. The overlap is one reason an accurate diagnosis can take careful work.

Alzheimer's disease

NIA identifies Alzheimer's as the most common form of dementia and describes brain changes involving proteins called amyloid and tau. In its [Alzheimer's explanation](#), NIA says memory problems can be an early sign, but difficulties with words, reasoning or judging what is seen can also appear early.

Ask the clinician which abilities are affected in your parent and how that fits the diagnosis. Remembering something well doesn't settle whether the disease is present.

Vascular dementia

"Vascular" refers to blood vessels. NIA explains that vascular dementia results from conditions that damage vessels in the brain or interfere with the brain's blood and oxygen supply. Its [vascular dementia page](#) describes possible difficulties with familiar tasks, following instructions and language, among other changes.

NIA also notes that not everyone who has a stroke develops dementia. So a stroke history is something for the doctor to assess, not a diagnosis the family can make. Ask what evidence points to a vascular cause in your parent's case and what existing health conditions the medical team needs to address. Don't infer a treatment plan from the word "vascular."

Lewy body dementia

Lewy bodies are abnormal deposits of a protein called alpha-synuclein. NIA's [Lewy body dementia guide](#) describes possible changes in alertness and attention, visual hallucinations, movement difficulties and sleep problems. It also says memory problems may not be apparent at first.

Looking only for forgetting could leave another concern unreported. Seeing something others don't see doesn't prove Lewy

body dementia. Describe what happened to the clinician and ask whether the team needs further specialist input.

Frontotemporal dementia

Frontotemporal disorders involve damage in the frontal and temporal parts of the brain. The National Institute of Neurological Disorders and Stroke, another NIH institute, explains in its [frontotemporal disorders guide](#) that early difficulties can center on behavior, personality or language. Memory may not be the first obvious problem.

If those are the words in your parent's report, ask which form the clinician suspects and what that means for support now. The family shouldn't have to force the description into a memory-loss pattern before taking it seriously. A new language difficulty or a marked personality change deserves a medical explanation, not an argument about whether the person is trying hard enough.

Mixed dementia

NIA describes mixed dementia as more than one type occurring together. For example, a person can have Alzheimer's-related changes alongside vascular changes. "Mixed" doesn't mean that the office has combined several relatives' guesses. It describes possible coexisting causes.

Ask which causes the clinician thinks are involved, and how certain they are. There may be more evidence for one component than another. An honest explanation of that uncertainty is more useful than a family-made label that sounds definite but has no medical assessment behind it.

When you write down the suspected type, keep the clinician's words about confidence beside it. "Likely" and "confirmed" aren't

interchangeable in your notes. If a term isn't clear, ask what it means in this report rather than supply your own meaning.

How the clinician puts the pieces together

According to NIA's diagnostic overview, an assessment begins by looking for underlying, potentially treatable conditions that could be contributing to difficulties with thinking. Medical and family history help, along with an examination and appropriate tests. Questions about when changes began are part of that work.

The same overview names medication side effects, vitamin deficiencies and thyroid problems among conditions that can cause serious memory problems resembling dementia. Mood conditions can also contribute to symptoms. These possibilities need clinical evaluation. Finding something treatable is not the same as proving that an established dementia has gone away.

I would resist both easy answers: "There's nothing to do" and "This one thing will fix everything." Ask what has been checked. A family member's guess isn't enough reason to change what your parent takes.

NIA describes cognitive tests that assess abilities such as memory, problem solving and language, while neurological testing can assess physical functions, including balance and reflexes. The tasks may seem small from the chair beside your parent. Their meaning belongs with the professional evaluating the whole picture.

If a result includes a score, ask what it contributes to the assessment and what else the clinician considered, rather than treating the number as a forecast for next Christmas.

Brain imaging may also be part of the evaluation. NIA describes CT, MRI and PET scans as tools for looking at brain structure, activity or particular changes, depending on the scan. Imaging can help identify problems such as strokes or tumors. These scans do

different jobs; a family shouldn't assume that the newest-sounding one is automatically the right one.

Other selected tests may involve blood or the fluid surrounding the brain and spinal cord. NIA's overview cautions that blood test results alone shouldn't be used to diagnose dementia. Ask what a proposed test is intended to answer in your parent's evaluation. Testing options and availability change, so confirm the current choices with the clinical team rather than use this book as a shopping list.

Not every procedure named on an information page belongs in every person's assessment. Ask whether a proposed test's result would change the recommendation. You aren't expected to order the workup yourself.

Primary care can be the starting point. NIA says the primary care doctor can help find a specialist, and names neurologists, geriatric psychiatrists, neuropsychologists and geriatricians among professionals who may diagnose dementia. A neurologist specializes in the brain and nervous system. A geriatrician focuses on older adults' health. The other specialists bring expertise in mental health or the assessment of thinking abilities.

Those titles aren't a ladder you have to climb in order. Ask the referring office which professional is suited to the question that remains. Chapter 11's written-observation page will help you prepare for that person's visit; you don't need to build it here.

Another opinion can have a specific purpose

You can respect a doctor and still ask for a specialist's assessment or another opinion. I don't regard that as disloyalty. The question is whether another assessment would help answer something important, not whether you have thanked the first doctor enough or waited a polite length of time to ask again.

NIA notes that similarities among dementia symptoms can make an accurate diagnosis difficult. That is a good reason to ask how confident the clinician is about the type. You might need clarification because the report names only dementia, because proposed testing hasn't been completed, or because an important change hasn't been explained.

Be specific about what you want reviewed. "Can another clinician help clarify the likely cause?" is more workable than "We want somebody to tell us everything will be fine." Another opinion may support the first one. It may leave uncertainty. It isn't a promise of better news.

Ask the office how to arrange the referral and share the existing results. If there's no appropriate specialist nearby, NIA suggests contacting the neurology department of the nearest medical school for referral help; a medical school hospital may have a dementia clinic. You can ask about that route without assuming a particular clinic will accept your parent or have an immediate opening.

While another appointment is being arranged, ask the current clinician what to do about unresolved health concerns and what follow-up is needed. Waiting for a specialist should not become an unattended stretch in which nobody knows who is answering a current medical question. Give the referral its owner and drop date from the introduction, but leave the clinical decisions with the medical team.

A stage is a description, not a date

The Alzheimer's Association's [stages guide](#) uses mild, moderate and severe, also called early, middle and late. It explicitly says these stages are general guides and can overlap. Its descriptions concern Alzheimer's disease; they are not a universal timetable for every cause of dementia.

In mild or early Alzheimer's, the Association describes people who may still function independently while having difficulties such as memory lapses or trouble planning. "Mild" is the stage label. It doesn't mean the diagnosis feels mild to you, or that an individual activity has been cleared as safe.

In moderate or middle Alzheimer's, difficulties become more pronounced and more assistance is needed. The Association describes trouble expressing thoughts and doing routine tasks without help. In severe or late Alzheimer's, it describes extensive care needs and major difficulties with communication and physical abilities.

Read those descriptions as an explanation of language you may hear, not a set of scenes you must rehearse tonight. You can ask what support is needed now without imagining every later possibility in detail. When a clinician uses a stage word, ask them to connect it to your parent's actual abilities.

Progression speed and the course of symptoms vary by person and by dementia type. The Alzheimer's Association says the rate of Alzheimer's progression varies; NIA's Lewy body guide also describes wide individual variation. Neither description gives you a calendar prediction for your parent. The date of diagnosis doesn't tell you how much assistance will be needed a year from that date.

I would be careful with advice that attaches a care decision to a stage word alone. Ask about the actual task and risk. The home-safety and driving questions in Chapters 13 and 14 need individual attention; "early stage" isn't permission to set them aside.

There is also an urgent limit to stage language. NIA distinguishes delirium, a sudden state of confusion, from the ongoing problems being assessed for dementia. MedlinePlus, the National Library of Medicine's health information service, says in its [confusion guidance reviewed in 2026](#) to call 911 or the local emergency number for

confusion that begins suddenly. Don't rename a sudden change "the next stage" and wait for an ordinary appointment.

The CDC's [stroke signs guidance, updated in 2026](#), gives the emergency rule. Call 911 for sudden confusion, new trouble speaking or understanding, one-sided weakness, sudden vision changes, or other possible stroke signs; note when symptoms began. For other changes that are not emergencies, contact the treating clinician. Their knowledge of your parent's history matters here. Chapter 15 addresses behavior changes in more detail; a book's explanation must never replace help for a current health concern.

Your diagnosis-visit question page

Use one sheet for the questions you need answered about this diagnosis. You can take it to the appointment or use it when asking the office for clarification afterward. Put your parent's name and the visit date at the top. Leave room for answers, rather than squeezing the questions between old notes.

This is a conversation aid, not a diagnostic checklist. Include your parent's own question if they want one asked. Mark the unanswered question that matters most to you, so an unfinished page still tells you what to pursue next.

- What is the diagnosis, in plain words, and what exact name should we use?
- Which dementia type or types do you suspect, and how certain are you?
- What findings support that conclusion, and what other causes or contributors have you considered?
- Is anything still being evaluated? What question would another test answer?

- Would a specialist assessment or second opinion help, and what should it clarify?
- If you use a stage label, what does it mean for the help my parent needs now?
- What medical options should we discuss for this diagnosis, and where can we read reliable information about them?
- Which changes require urgent help? Who handles other questions, and what is the follow-up plan?

Keep answers separate from guesses. If the clinician says the type remains uncertain, write that. For a pending test, record who will explain its result rather than filling the empty space with a confident sentence borrowed from another family's experience.

You don't have to get through every question in one sitting. Start with what's unresolved. An appointment on the calendar and an answer to a pending question are not necessarily the same thing.

Treatment recommendations belong with the clinician who knows your parent's circumstances. This question page doesn't recommend a medicine, supplement or treatment change.

Keep the written page where you'll find it for the next conversation, not buried under the appointment card. Tonight, underline the question that still needs an answer. If you can't answer the father's question from the opening — which diagnosis did they give? — contact the office and ask for that clarification first. Leave the pen beside the page.

Chapter 11: Getting the Most From Doctor Visits

Mama had folded her coat across her knees. When the doctor asked how she was doing, Dot said, “Doing fine.” I sat beside her with a notebook still in my pocket. There were things I had meant to mention about the nights at home. I couldn’t find a way to begin without sounding as though I had come to correct her.

The visit lasted ten minutes. We left with the things I needed help with still unsaid.

In the car, I was angry with the visit. Later I had to admit that the doctor couldn’t read the page I hadn’t shown. Mama had answered the question she’d been asked. My notebook hadn’t left my pocket.

I’ll tell you what helped: sending a short written account before the next appointment, then bringing a copy. No speech required. The observations could reach the person who could assess them even if my nerves got the better of me again.

This week’s job is to prepare that account and a current health summary. One describes changes since the last conversation. The other gives a new provider basic health information without requiring you to reconstruct it across a desk.

Get the concern into the room before you do

Ten minutes is a poor container for everything a family has been carrying. A polite opening, an examination and a closing question can leave something important outside the conversation, without anyone being careless.

I wouldn't make your parent's answer the obstacle. "Fine" might mean that nothing hurts just then, or that they would rather be somewhere else. Their account and yours can both be heard.

The National Institute on Aging's [appointment-preparation guide](#) recommends writing down concerns, putting them in order and raising the most important ones first, rather than saving the matter that brought you there for the hand already on the door handle.

Choose a main question. "We need help deciding what to do about the change in her sleep" gives your notes a purpose. "We need to talk about everything" doesn't. Mention other concerns too, and let the clinician determine the clinical priorities.

Before sending health details, ask the office how it accepts a family member's observations. It may have a patient portal, a form or another approved route. Ask whose attention to mark the note for and when it should arrive. Don't assume every office needs the same advance notice.

Use the access arrangement the office approves, not your parent's password borrowed without permission. Ask separately what is needed for the office to discuss care with you. Delivering a note doesn't settle that question. The legal-authority questions belong in Chapter 16; here, you need to know how this office handles communication.

Nor would I promise your parent that a note is secret; ask the office how it will be used and who may see it. If discussing a particular concern in front of your parent feels difficult or unsafe, tell the staff that and ask how to raise it appropriately. Don't improvise a private channel and assume it stays private.

If electronic access isn't workable, ask about a paper route. Keep a dated copy. At the appointment, ask whether the clinician has received the note, and offer your copy if needed rather than relying on a message you can't confirm was read.

Record changes, not a verdict

Between appointments, keep brief dated notes about changes you need help understanding. Give the clinician something more precise than “She’s worse.” You aren’t conducting a home medical assessment or watching every ordinary afternoon as though it were an examination.

Before any tracking, keep the emergency boundary from Chapter 10. The CDC’s [stroke guidance, updated in 2026](#), directs immediate 911 for sudden confusion, sudden problems producing or understanding speech, abrupt vision changes, weakness on one side, sudden loss of balance, or a sudden severe unexplained headache. Note when the symptoms began, but don’t delay the call to finish a record. MedlinePlus’s [confusion guidance reviewed in 2026](#) also directs emergency help for confusion beginning suddenly. Don’t wait for appointment day.

For changes that aren’t emergencies, contact the treating clinician about when assessment is needed; don’t assume they can wait until the next scheduled visit. Ask what to report sooner and which number to use outside office hours. The clinician needs your parent’s history to judge the concern. These notes can’t make that judgment for you.

Start with sleep. Write what you actually know: when you noticed your parent awake, what they were doing, and whether you were there to observe the rest of the night. “Awake when I checked at midnight” doesn’t establish that they never slept; if you weren’t staying overnight, say so instead of filling the gap with a guess.

NIA’s preparation guide asks patients to report sleep changes, along with changes in appetite, weight and energy. If the clinician wants a particular kind of record, ask them to show you what would be useful.

For a fall, record when and where it happened and what you saw, keeping your account separate from your parent’s if they told you

about a fall you didn't witness. Record any medical help already sought and the instructions received. Don't turn an unwitnessed fall into a confident explanation about a rug, a shoe or the dementia itself. If your parent is hurt or cannot get up after a fall, call 911 for help. After a blow to the head, worsening headache, repeated vomiting, new confusion, unusual drowsiness, slurred speech or weakness needs immediate emergency care; don't wait to finish a note or for the appointment. If you suspect a concussion without those signs, contact a health care provider promptly. Tell the treating clinician about every fall, even when no injury is apparent.

These boundaries come from NIA's [falls guide](#) and the CDC's [head-injury danger signs](#) and [concussion guidance](#). Let the treating clinician determine the evaluation needed, especially when a head injury or blood-thinning medicines are involved; don't try to diagnose the injury at home.

For confusion that has changed gradually, describe the task that became harder and when you first noticed. "Needed help finding the bathroom at home" says more than "Memory bad." The clinician can assess the meaning; you aren't assigning a stage. Use the diagnosis questions from Chapter 10 if needed; Chapter 15 addresses behavior patterns.

Be honest about how often something happened. "I saw it once" is usable. Once is enough to report. "Every day" should mean that you have evidence for every day, not that the incident stayed in your mind all week. If another person told you about it, name that source in the note to the clinician.

Put medication changes alongside the observations, with dates when known. Copy the actual name and directions; "the little white one" won't identify what the prescriber changed. Write who ordered the change and whether you know when your parent started following it. A prescription date and the date the medicine was first taken needn't be the same.

A sleep change after a new prescription is a reason to ask the prescriber, not evidence that you have found the explanation just because one event followed the other. NIA's [medicine-safety guide](#) advises reporting suspected medicine problems promptly and discussing them with a provider before stopping a medicine. Leave medication decisions with the prescriber; ask a pharmacist when you need help identifying a medicine or understanding its instructions.

Don't make up missed-dose instructions, and don't change a dose to test your theory. Ask the prescriber or pharmacist what to do. In a possible emergency, use emergency help rather than waiting for an ordinary medicine question to be answered.

Keep raw notes short enough to continue on a busy day. For the visit, select the observations relevant to the main concern. Provide the whole record if requested, but don't hide the question under weeks of detail.

Give the next provider a current starting point

A visit note expires as its question gets answered. A health summary travels. Start with one page, dated at the top, and attach a clearly labeled medicine list if it won't fit legibly, rather than leave out a medicine to meet a page limit.

Include your parent's name, date of birth, the diagnosis as the clinician recorded it, other known health conditions and relevant operations or hospital visits. Preserve the confidence language from Chapter 10 for an uncertain diagnosis.

Add allergies and known medication reactions as recorded by the health team, along with the names and numbers of the current clinicians and pharmacy. Unknown? Mark it for checking. Guessing an allergy history to fill a blank is worse than leaving a visible question for the office to resolve.

NIA's medicine-safety guide recommends listing prescriptions, over-the-counter products, vitamins and supplements, including the amount and times taken. For prescriptions, include the prescriber and reason for use. Include occasional products too, not just the bottles lined up beside breakfast. Tell the clinician when the reason for a medicine isn't known.

Compare the list with the containers and the latest instructions from the medical team. If they disagree, flag the discrepancy and ask the prescriber or pharmacist to resolve it; don't choose which instruction wins yourself. Mark uncertainty about what was actually taken rather than presenting the label as proof that every dose was taken.

Add how your parent manages ordinary tasks now: what they do independently and what help they receive. Note hearing or vision difficulties. "Daughter helps communicate questions" describes assistance without declaring that your parent has nothing to say.

Put the agreed family contact and their phone number on the page, subject to the office's communication arrangements, and keep this health information out of public calendars or casual group messages. The appointment card can say where and when; it doesn't need the medicine list beneath it.

NIA's appointment guide recommends bringing insurance cards, other doctors' contact information and medical records the new doctor doesn't already have. Ask which records the office needs and how to arrange their transfer. Your page can help orient a new provider; it cannot replace the underlying reports or their review.

Update the summary after a verified change. Date it and replace the copy in the appointment folder. Agree who updates it; you don't need several competing lists maintained with equal enthusiasm.

Leave your parent a place in the conversation

Before the visit, ask what your parent wants to raise. Put that concern on the page too. You may be attending because you can remember details and follow up, but it is still their appointment. NIA's preparation guide cautions companions against taking too strong a role and notes that patients may want time alone with the doctor.

Tell the staff what help your parent needs to participate. Bring their glasses and working hearing aids if used, as NIA recommends. Ask ahead for an interpreter when needed. These are practical arrangements for the visit, not a test of how well your parent can manage without them.

Allow space for your parent to answer. Add observations without turning the appointment into a contest over who remembers correctly. "I'd like to add what I noticed at home" is enough. For help with difficult conversations, turn to Chapter 12.

With your parent's agreement where they can give it, confirm who is authorized to receive health details and share only the current information that person needs for the visit, through an agreed private route. Ask the office about the right arrangement when your parent cannot decide or objects; don't pass a whole medical page to someone merely because they are attending. Say which unanswered concern matters, and leave room for the reply. Don't depend on a car conversation to deliver the entire health history to a sibling who hasn't attended before.

The U.S. Department of Health and Human Services explains the distinction between sharing information with [involved family and friends](#) and the right to access [medical records](#); being related or attending a visit isn't blanket access.

At the start, tell the clinician about the main concern and any other items you brought. Ask what can be addressed today and what needs another appointment or a different service. If care costs are

the unanswered issue, Chapter 17 covers that ground; this page need only record whom the office suggests contacting.

When an instruction isn't clear, ask for it again in plain words, then read back what you have written so the clinician can check whether you've understood correctly. For a medicine instruction, clarify the exact product and directions rather than settling for "continue as before" when you don't know what "before" refers to.

You also need to know how an unfinished matter will be answered. For a test, ask who explains the result and when to expect contact. Ask what to do if that time passes without an answer. For follow-up, write the proposed timing and who arranges it. A result appearing online doesn't by itself answer your question about what happens next.

The before-the-visit observation page

Use the following headings on a sheet of paper or in the office's approved message format. This is the change-focused note, not the health summary or Chapter 10's diagnosis question page. Write only what you know, leaving uncertainties visible. Keep the completed note to a readable page where possible, with further details available if requested.

- **Parent and appointment:** Name, date of birth, visit date and clinician. Your name, relationship and contact details; date of this note.
- **Main concern:** The change you want assessed and the question you need answered. Add your parent's concern in their own words where possible.
- **Before this change:** A brief description of the relevant task or sleep pattern before things changed. State what you personally knew.

- **Observed changes:** Dated examples involving sleep, falls or gradually changed confusion, as relevant. Say what happened, who observed it and what isn't known.
- **Medicines or other recent care:** Verified prescription changes and start dates, new nonprescription products, emergency visits or other care. Attach the current medicine list rather than guessing names.
- **Help already sought:** Whom you contacted, when, the advice received and any question still awaiting an answer.
- **What we need from this visit:** The main question, any other concerns to flag, and a request for the next step and follow-up contact.

This form isn't for storing a sudden change until appointment day. Use the emergency boundary above and the clinician's reporting instructions before completing paperwork.

After the appointment, add the clinician's answer and the agreed next step beneath the question, keeping their recommendation separate from your own interpretation so another person can see what was actually advised. If you couldn't get an answer, write that too, along with whom you will contact and when. Apply the owner and drop date from the introduction to that follow-up, not to a medical decision the family can't make.

For the next visit, open a fresh page. Carry forward an unresolved question, not every paragraph from the old one. Mama's "doing fine" wasn't a reason for me to have nothing ready. Put your parent's name at the top tonight, write the change you need help understanding, and leave enough white space for someone to answer.

Chapter 12: Talking With Someone Who Is Confused

Seven in the morning, and the same question for the fourth time.

Here is a made-up kitchen, a son and his mother. “What time is the doctor coming?” The appointment is Thursday at ten. He tells her Thursday at ten. She nods, stirs her coffee, looks out at the carport, and eleven minutes later asks what time the doctor is coming.

By the fourth asking his voice has changed. He doesn’t yell. He just lands on the word *told*. “Mama, I’ve told you three times.”

She goes quiet after that. Not because the answer finally stuck. Because she heard how he said it.

That’s the part that gets missed. The information drops away first; the tone stays in the room a good while longer. You can be right about Thursday and still lose the morning.

This chapter is about the ordinary talking: answering, asking, explaining, getting through a task together. Accusations and suspicion are a different animal, and they wait for Chapter 15.

Correcting costs more than it buys

My instinct, early on, was to fix every wrong detail. The wrong day, the wrong grandchild’s name, the wrong year for a trip. I believed I was being helpful, and some part of me believed that if I kept the facts straight out loud, the facts would stay straight in her head.

They didn’t. What I got instead was a woman who stopped offering sentences, because every sentence had turned into a quiz she

kept failing.

I'll tell you what helped. I let most of it go.

The Alzheimer's Association's guidance on [memory loss and confusion](#) suggests staying calm, responding with a brief explanation rather than a long one, and offering corrections as suggestions instead of explanations that sound like scolding. Its example is small and useful: "I thought it was a fork." The Association also notes that in a conversation anchored in another time of life, you can travel there with the person and talk about what they recall, treating it as their current reality. And it says plainly that being called the wrong name, or not being recognized, is not something to take personally, however much it stings.

Its [communication guidance](#) puts the same idea in two lines worth keeping on a card: avoid criticizing and correcting; avoid arguing, and if the person says something you don't agree with, let it be.

That's the opposite of advice families often get, which is to reorient the person to today's date and today's facts every single time. I'd be careful with that one. Insisting on reality doesn't restore it, and if the only reliable product of a correction is distress, you've bought nothing.

There's a line I won't draw for you, because I can't. What to say when your mother asks for a husband who died in 1998 is the hardest question in this book, and families settle it differently, some with a brief honest answer, some by asking what she remembers about him. Ask your parent's care team what they'd suggest, and ask your local Alzheimer's Association chapter; they've heard the question from people in your exact spot.

One thing stays fixed whichever way you go. Talk with your parent as an adult, not as a child. The National Library of Medicine's MedlinePlus guide to [dementia daily care](#), reviewed in April 2026, says that outright: speak to people who have dementia as adults, and

don't pretend to understand them when you don't. Baby talk is a correction too. It just arrives in a kinder voice.

Set the room before you set the sentence

Half of what goes wrong in these conversations was decided before anybody opened their mouth.

The MedlinePlus daily-care guide recommends cutting distraction and noise first: turn down or turn off the radio or television, close the curtains, move to a quieter room. It suggests making eye contact before you touch or speak to the person, so you're not arriving out of nowhere.

Then the sentence itself. That same source recommends simple words and short sentences, spoken slowly, in a quiet voice. Volume isn't the fix. Talking loudly, as though the person were hard of hearing, doesn't help if hearing isn't the problem. Repeat your words when you need to. Use the names of people and places your parent knows, and go easy on pronouns, since *he*, *she* and *they* can leave a listener hunting for who's being discussed. If you change the subject, say that you're changing it.

Two more, from the Association's communication guidance: speak directly to the person rather than to whoever brought them, and don't leave them out of conversations happening in front of them. If a pharmacist asks you how your parent is sleeping while your parent is standing there, you can bring them into the question: 'Mama, do you want to tell her about the nights?'

The Association also suggests asking what the person is still comfortable doing and where they'd like help, and agreeing on which way of communicating suits them best, whether that's sitting face to face, a phone call or something written. Ask early; you may need to ask again as your parent's preferences or abilities change.

And timing counts. A question that goes nowhere at six in the evening may land fine at ten in the morning. If you've got something that genuinely needs discussing, take it to the hour of day your parent does best, whatever hour that turns out to be. Why late-day hours run rougher for some people is Chapter 15's subject, not this one.

Sensory trouble muddies all of it. The Association notes that vision and hearing loss may also be present in someone with dementia, which makes communication harder still. It adds that many such problems can be addressed, with hearing aids or prescription lenses, and recommends having hearing and eyesight checked regularly and asking your parent's doctor whether either is affecting communication. Before you conclude that your father has lost more words this month, find out whether his hearing aid has a dead battery. Add the hearing and vision question to the summary you built in Chapter 11 and let the clinician sort out what's what.

One thing at a time, and the time to answer it

Most of us ask stacked questions without noticing. "Do you want to get dressed now or after breakfast, and did you want the blue shirt, and should I run the bath first?" That's three decisions and a plan, delivered in one breath, to someone who may still be working on the first word.

The Association's middle-stage communication tips go the other way. One question at a time. Yes-or-no questions where you can: "Would you like some coffee?" rather than "What would you like to drink?" Clear, step-by-step instructions, since a long request can be too much to hold. Visual cues, including showing a task rather than describing it. And written notes, which can land when spoken words don't.

MedlinePlus adds the same approach from its side: break directions into small, simple steps, allow time for the person to understand, offer clear choices with something to point at, and don't pile on options. If frustration sets in, it suggests switching to another activity rather than pushing through.

Then the hardest instruction on the list. Wait.

The Association recommends giving the person plenty of time to respond and not interrupting unless help is asked for. A pause that feels endless to you is often just the time a sentence now takes to assemble. Count it out if you have to. Finishing your parent's sentences is efficient, and it teaches them that talking is something other people do faster.

Keep one thing straight about all these adjustments. They make a conversation work; they don't measure what your parent can still do. A father who follows a two-step instruction in a quiet kitchen has not thereby been cleared to drive, manage his own medicines or stay alone overnight. Those are clinical and safety judgments, and they belong with his doctor and the chapters that deal with them.

When the same question comes back

Treat a repeated question as a fresh chance to respond, without assuming why it returned. Answer briefly; if the worry remains, ask what feels uncertain.

For the question in the kitchen scene, the uncertainty might be about what happens next, not just the time. The Association, writing about late-stage communication, makes the general point: the feelings behind the words can matter more than the words. You can add a concrete detail to the answer: "Thursday. I'm driving you, and I'll be here early."

Written notes earn their place here. A card by the chair that says *Dr. Pham, Thursday 10 a.m., John driving* gives your parent

somewhere to go besides you. Some people check the card a dozen times and settle. Some never look at it. You'll find out which you've got.

Don't buy quiet with a promise you can't keep. "We'll go see the old house tomorrow," said to end a repeated question, buys you about a day and costs you the next conversation.

Repeated stories are a lighter problem, and I'd treat them lightly. If your mother tells you about the ice storm in Statesville for the third time this week, you're not obliged to log it as a symptom or to announce that you've heard it. Let it run. Ask what the roads were like. You are allowed to simply enjoy your parent.

The Association's early-stage guidance makes a point about this that I'd underline for every relative who has gone quiet since the diagnosis: don't assume what someone can or can't do with a conversation because of a diagnosis, and don't pull away. It says the disease affects each person differently, and that your honesty, friendship and support still matter. It even says that laughing is allowed, that humor sometimes lifts the mood and makes talking easier. My mother said funnier things in her last few years than she had in the twenty before, and laughing with her was never the problem. Laughing at her would have been.

This is also the thing to tell the grandchildren and the brother-in-law who keeps finding reasons not to visit. They are usually not avoiding your parent. They're avoiding the fear of saying the wrong thing. Hand them two sentences and a job, something ordinary with a start and a finish, and most of them manage fine.

There is a line where a communication technique stops being the right tool. The Association's memory-loss guidance says that any sudden change in behavior should get a medical evaluation to rule out other causes, and lists infections, a change in living arrangements and a switch in routine among the things that can make symptoms worse. A parent who was managing conversation

last week and can't today needs a clinician, not a better phrase. New trouble producing or understanding speech that comes on suddenly is an emergency: call 911, as the CDC's stroke guidance directs, and note what time it started.

Safety also outranks technique. MedlinePlus's guide to [dementia behavior and sleep problems](#), also reviewed in April 2026, advises against touching or restraining someone during an angry outburst unless safety requires it. It suggests staying calm, trying to distract the person, and not taking the behavior personally. And if you or your parent is in danger, that guide says to call 911 or your local emergency number. Don't stay in the room trying a better phrase. That instruction comes before every piece of advice in this chapter.

A say-this-not-that card

Copy these onto an index card and leave it where the hard conversations happen, which for most families is the kitchen. They're swaps, not scripts. Your mother's name and your father's history go in them, or they'll sound like somebody else talking.

- Instead of "I already told you, it's Thursday," try: "Thursday at ten. I'm driving you."
- Instead of "Don't you remember your own granddaughter?", try: "That's Julie, your granddaughter. She's got your eyes."
- Instead of "That's not a fork, it's a spoon," try: "I think that's the spoon. Here's the fork."
- Instead of "Mama, Daddy died years ago," keep this reminder: "When your parent asks for someone who has died, don't borrow a stock answer from this card. Acknowledge the feeling, notice how your parent responds, and ask their care team what approach fits this person."

- Instead of “Do you want a shower now or later, and which shirt?”, try: “Would you like a shower now?” and ask about the shirt afterward.
- Instead of “Hurry up, we’re late,” try: “We need to leave soon. Let’s do one thing at a time.” Then say nothing and let the sentence arrive.
- Instead of “You can’t do that anymore,” try: “Let me get the first part started.” Leave the clinical judgment to the clinician.
- Instead of “What did you do today?”, try: “Did you sit out on the porch today?”
- Instead of talking over your parent to another adult, try: “Mama, do you want to tell him, or should I?”

Some swaps will sound natural to your family; others won’t. Keep what helps and revise what doesn’t.

What to try this week

Pick one sentence you keep saying that keeps going badly. Mine was a version of “I just told you.” Write down what you’ll say instead, in your own words, and put it somewhere you’ll see it before your parent is standing in front of you.

The goal isn’t a perfect conversation. Some days the exchange that matters is three words long and mostly about the fact that you sat down.

Chapter 13: Home Safety and Wandering

The screen door catches on its spring. The daughter comes out of the kitchen.

In this imagined near-miss, her father is on the porch in his slippers, one hand on the railing. His coat is still on its hook. He says he needs to get to work. The daughter had stepped away to rinse a plate; she thought he was watching television.

She goes to him. They come back inside together. Nothing terrible has happened, and her hands are shaking anyway.

Later, her brother says they ought to get a better lock.

A door that your father cannot open can keep him inside during a fire, too. You need a way to notice an exit, someone who can respond, and a usable escape route.

This week's job is a safety pass through the house and a written plan for a missing parent. If your parent is missing now, call 911 and tell the dispatcher they have dementia. Don't finish reading first.

Walk the route your parent actually uses

Start at your parent's chair and follow the route to the bathroom. Then walk from the bed. Look at the floor, the light switches, the narrow turn beside the dresser. A tidy-looking house can still have a cord across the path someone uses in the dark.

The Alzheimer's Association's [home safety guidance](#) recommends removing throw rugs, extension cords and excess clutter from walking routes. It also calls for even lighting between rooms and at

stairs and entries, with night lights in bedrooms, bathrooms and halls.

Put the reading lamp's cord against the wall rather than across the trip to the kitchen, and move the shopping bag waiting beside the chair before you check the route again after dark.

No risky demonstration needed. Check the steps themselves.

Remove the obstacle without making a trial of who left it there.

The Association says a home should support independence and social interaction rather than feel too restrictive. Clear a space at the table without stripping the room of everything your parent enjoys.

The same guidance recommends securing large televisions, cabinets and bookcases against tipping. If you don't know how to anchor an item properly, get qualified installation help.

In the bathroom, the Association recommends grab bars by the shower, tub and toilet, and nonslip treatment for slippery surfaces. Ask an occupational therapist or home-safety professional which supports fit your parent's abilities and how they should be installed. Don't assume a towel rail is a grab bar. The bathing routine itself belongs in Chapter 15a; here, you're checking the room around it.

The fall-and-head-injury boundary in Chapter 11 still applies. Tell the treating clinician about falls and ask for an individual safety assessment when walking, vision or transfers have become difficult. Changing the room doesn't replace that assessment.

Before buying anything, name the problem. "Can't see the step from the hallway" gives you something to ask about. "Need all the dementia safety products" doesn't.

The kitchen needs more than a reminder

A note saying *Turn off the stove* may not be enough protection when using the stove has become unsafe. You need to address access to the hazard, not keep making the note larger.

The Association's home safety page suggests appliances with automatic shutoff and measures to prevent unsafe stove use, including knob covers or removing knobs. Ask a home-safety professional about the appropriate arrangement for the appliance and your parent's current abilities. Gas or electrical changes belong with a qualified professional; don't improvise a disconnection because an online list mentions it.

An automatic feature doesn't settle whether your parent can use the appliance alone, so discuss that question with the care team before treating the gadget as permission to leave. Until then, provide the needed help.

Look at what is sitting beside the kettle. The Association recommends taking medicines and vitamins off counters and tables and storing medication in a locked drawer or cabinet. Arrange safe access for the person responsible for giving it. This is storage, not permission to change a medicine or its directions; keep the prescriber-and-pharmacist boundary from Chapter 11.

In the laundry area, secure cleaning products out of sight in their original containers. The Association specifically warns about laundry packets and bleach. A decorative jar makes a poor place for something that isn't food. Check the garage and workroom for accessible chemicals or equipment as well. Don't confine your parent to secure a bottle; secure the bottle.

Put smoke and carbon monoxide alarms on the safety pass. The Association says these devices should be available and inspected regularly. If there are firearms in the house, its guidance recommends removal or locked storage. Ask for qualified help with safe handling or storage rather than handling an unfamiliar firearm yourself. This chapter isn't a firearm-handling guide.

Notice risk before an actual disappearance

In its [wandering.guidance](#), checked in October 2026, the Alzheimer's Association states that six in ten people living with dementia will wander at least once. This is its currently published estimate, not a newly dated study or an individual prediction.

The Association says wandering can happen at any stage. Use the figure as a reason to prepare, not a reason to watch your parent as though every trip to the porch were wrongdoing.

The warning signs on that page include taking longer than expected on a familiar walk, forgetting a familiar route, pacing, and appearing lost in changed surroundings. A person may talk about an old job or say they need to go home while sitting in their own house. Difficulty finding rooms at home is another sign the Association names.

Those signs give you something to discuss with the care team. They don't prove that every restless afternoon will end in a disappearance. Nor does the absence of a previous incident make a missing-person plan unnecessary.

After a near-miss, contact the treating clinician promptly about what happened and what supervision is needed now. Don't wait for a repeat to justify the call. Sudden confusion still takes the emergency route from Chapter 10; don't file it under wandering and wait for a safety appointment.

A parent can also get lost by car. Once driving has stopped, the Association recommends removing access to car keys. Deciding about driving and arranging that transition belong in Chapter 14. Here, check that an agreed restriction isn't undone by the spare keys on the hall hook.

The wandering guidance recommends areas that can be explored safely, indoors or outside. Ask the care team how to support that for this person. The aim isn't to eliminate every enjoyable movement.

An alarm must have someone at the other end

A door alarm tells you a door has opened. It doesn't bring your parent back.

The Association lists warning bells and door-monitoring devices among measures to consider. Before relying on any door alarm, monitoring device or lock placement, have your parent's doctor, the local fire department, or an occupational-therapy or home-safety professional review the setup. Explain who is present during the day and overnight, because an arrangement built around an awake companion in the house is different from one depending on a phone notification reaching somebody elsewhere.

Ask who will hear or receive an alert, and who can respond in person. Try the agreed system while your parent is safely accompanied, from the places the caregiver actually occupies. If it uses a phone, check its sound and notification settings. Ask the provider what happens during loss of power, an internet outage or a depleted battery. Write the backup arrangement, rather than assuming the device will announce its own failure.

If nobody can respond promptly, tell the care team that. Don't substitute a lock or a promise to check the screen more often for missing supervision. If there is immediate danger and you cannot keep your parent safe, call 911 for help.

GPS equipment deserves the same skepticism. The Association's [technology safety guidance](#) says tracking can be inaccurate, may work poorly inside buildings, and should be used alongside other safety measures. It is not a companion. A location dot may help a search; it isn't proof that a person is safe where the dot appears.

Ask how a device stays with your parent and who keeps it charged. Talk with your parent about monitoring where they can participate, and ask the care team about privacy and consent questions if that is difficult. Don't quietly turn every room into a recorded space because a camera was easy to order.

The Association's wandering page also suggests deadbolts above or below the usual line of sight. In the same instruction it says never to lock a person in at home. Read those together. This is not a recommendation to install an unreachable bolt yourself. If a proposed lock prevents your parent from getting out, reject that arrangement and ask for another approach.

Every emergency exit must remain usable by your parent and the caregiver, with assistance arranged where needed; first responders must be able to reach them and use the escape routes. A caregiver's hidden key is not an escape plan for a parent alone behind a locked door. Neither is an app that has to connect before the door will open.

The U.S. Fire Administration's [home fire escape guidance](#), checked in 2026, calls for unblocked doors and windows and an outside meeting place. Have the local fire department review how your household would get out, including your parent's actual need for help, and ask about local requirements before changing door hardware. A dementia label on the package isn't enough.

I wouldn't buy a door device before making that call. The question isn't just whether it keeps someone from leaving. It's whether everyone can leave when they must.

A room-by-room safety pass

Never lock your parent in. Keep emergency exits usable for your parent, caregivers and first responders. Have any alarm, monitoring device or lock placement reviewed before relying on it.

Walk through with this checklist. Mark an item complete only when the arrangement works; write who will handle anything unfinished. A professional review still pending is an open task, not a checked box.

- **Walking routes:** Remove loose rugs, bags and trailing cords; check even lighting from chair and bed to bathroom, at stairs and at entries.
- **Bedroom and sitting room:** Check night lighting, accessible needed items and large furniture secured against tipping; seek help for installation or mobility questions.
- **Bathroom:** Identify slippery surfaces and needed grab bars; have suitable supports properly assessed and installed without blocking access.
- **Kitchen:** Address unsafe stove access with qualified help; check appropriate shutoff features and safe, secured medicine storage without changing treatment directions.
- **Laundry, garage and workroom:** Secure cleaning products in original containers; restrict access to hazardous chemicals and equipment, not escape routes.
- **Exterior doors and yard:** Arrange reviewed alerts with an actual responder and backup; check all exits remain usable. After driving stops, secure every accessible set of car keys.
- **Fire and other hazards:** Check smoke and carbon monoxide alarms; ask the fire department about escape assistance. Arrange qualified help for any firearm safety concern.
- **Caregiver handoff:** Show the next caregiver the reviewed setup, needed access and backup arrangements; put the missing-person card somewhere they can find it.

Bring an unhelpful change back to the professional who reviewed it. Recheck after a new safety incident or a change in abilities; a completed pass describes today's house, not every future need.

When you cannot find your parent

Prepare the information before you need it. The Association recommends keeping a recent close-up photograph, identifying dangerous areas nearby, and listing places your parent might head toward, such as a former workplace or place of worship. Ask trusted neighbors to contact you if they see your parent apparently lost. They are extra eyes, not a replacement for needed care.

Keep the photo available to responders and the card accessible to an agreed caregiver. Don't post private health details publicly as a precaution. Ask local police through their non-emergency contact about dementia-response services and the information they need. Availability varies; registration doesn't replace 911.

The Association's wandering page says to begin searching immediately and call 911 if the person hasn't been found within fifteen minutes. That instruction is not a required waiting period. For this book's conservative action card, call 911 as soon as you realize your parent with dementia is missing. Report an unknown departure time as unknown. Tell the dispatcher about immediate hazards, including traffic, water or dangerous weather, and follow their instructions.

Look in the immediate surroundings without delaying the call or putting yourself in danger. If another adult is available, one can speak with the dispatcher while the other checks nearby. Tell responders about the places on your list and any location-device information; don't treat a tracker as a reason to postpone help. Don't enter water or another dangerous area yourself.

Give the last confirmed sighting, not the time you wish they had left. "I saw him in the sitting room before I went into the kitchen" is an honest starting point. Distinguish what you saw from what a neighbor reported. Keep your phone available for responders, and tell them immediately if your parent returns.

Your missing-person action card

Write these details on one page now. This is an emergency aid, not another health-summary template.

1. **Call:** 911 as soon as the parent with dementia is missing. State the dementia diagnosis, their name and the address or location they disappeared from. Follow dispatcher instructions; don't wait fifteen minutes.
2. **Last confirmed sighting:** Time and place, or clearly marked uncertainty. Current clothing, footwear and any mobility aid, if known.
3. **Identification:** Recent close-up photo and an accurate physical description. Contact number for the caregiver speaking with responders.
4. **Nearby risks and possible destinations:** Traffic, water or other hazards; former addresses, workplace, place of worship and previous destinations, if relevant.
5. **Search coordination:** Who is checking the immediate vicinity, who remains reachable, and any tracking information. Do not delay emergency contact or enter dangerous areas.
6. **When found:** Notify responders immediately, follow their instructions and report any injury or sudden confusion. Contact the treating clinician about the incident and review the safety arrangement before using it again.

Nobody has to prove they were a good caregiver before making that call. Give the dispatcher the facts you have. Tonight, take a current photo and put it where the next person helping you can find it without asking.

Chapter 14: The Driving Conversation

Daddy answered the phone, and a deputy asked whether he was Dot's husband.

Mama had gone for groceries. Coming home, she had lost the route she knew. The deputy called Earl, my father, so someone could come for her.

That detail stayed with me. We had treated familiar roads as protection. They weren't enough.

In this composite family story, the call made the driving question impossible to put off. I wish we had raised it before somebody had to find Daddy's number. You don't need that call to ask for help.

This week's job is to bring driving to the treating clinician and check your state's licensing rules. If your parent is already driving against medical advice, address access to the vehicle now, safely, while getting professional help. An appointment weeks away isn't a safety arrangement for today's trip.

A familiar road still asks for quick decisions

The National Institute on Aging's [Driving Safety and Alzheimer's Disease](#), checked in October 2026, says some people can still drive in early dementia. Worsening memory and decision-making can make driving unsafe. The word *early* isn't permission. Your parent's individual assessment and your state's rules matter.

Knowing the way to the store doesn't settle how a driver will respond when a child steps off a curb. NIA says someone with

memory loss may manage some driving yet struggle to react quickly to a surprise. The question is today's decisions, not yesterday's driving record.

The Alzheimer's Association's [Driving and Dementia](#) describes difficulty with complex tasks such as driving even at the earliest stages of Alzheimer's. It recommends planning before driving becomes an issue. A diagnosis visit is a reasonable place to start that conversation, even without a crash.

I wouldn't begin with how old your parent is. NIA's [safe-driving guidance for older adults](#) recommends discussing driving skills rather than age. A lifetime of being the person everyone trusted behind the wheel deserves respect. It cannot answer a current safety question.

Signs to bring forward, not a score to pass

The Association lists forgotten routes, missed signs, pedal confusion and poor traffic decisions among unsafe-driving warnings; NIA also names new damage and reports from others. Use this checklist to identify a concern, not calculate a cutoff.

- Getting lost on a familiar route, forgetting the destination, or returning unusually late from a routine errand without a clear explanation.
- New dents or scrapes, hitting curbs, a near miss, or a crash.
- Missing stop signs, traffic lights or other road signs; errors at intersections.
- Confusing the accelerator and brake.
- Poor lane control, abrupt lane changes, inappropriate speed or slow decisions in traffic.
- Becoming confused or angry while driving, or receiving a specific report of unsafe driving from someone else.

- A clinician’s instruction to stop driving, or a failed driving evaluation.

An unexplained dent isn’t a confession. Ask what happened. Get other people’s accounts in their words, separate from what you witnessed, and use the Chapter 11 note route rather than making your parent defend their whole driving history.

Don’t wait for several boxes to be checked. With a warning of unsafe driving, arrange another driver and contact the treating clinician promptly about the event and what assessment is needed. Confusing pedals or missing a signal is not a reason for one more family test drive. Don’t put yourself in the passenger seat to collect proof of a danger you already suspect.

If confusion has begun suddenly, use Chapter 10’s emergency boundary: call 911. Don’t turn a sudden medical change into an ordinary driving appointment.

Ask what the doctor is assessing

Say the question directly: “We need a clinical driving-safety assessment. What should happen about driving now?”

If your mother got lost, say where she meant to go, where she was found and who helped. “Nobody saw the turn she missed” is useful uncertainty. Don’t fill it with a guess.

Ask the clinician to consider the health issues that could affect driving. NIA names vision, hearing and movement problems in its dementia-driving guidance. Its older-driver guidance also explains that medicines can cause drowsiness or reduced alertness. Give the doctor the current medicine information from Chapter 11; don’t stop a prescription to see whether driving improves. Medicine decisions still belong to the prescriber.

An office assessment is not the same thing as a formal on-road driving evaluation. A discussion or test in an examination room

doesn't mean anyone watched your parent respond to traffic. Ask which kind of assessment the office can provide, what its conclusion covers, and whether a separate driving evaluation is appropriate. Don't assume that a referral slip settles safety while you wait.

The Association recommends a comprehensive evaluation by an occupational-therapy driving rehabilitation specialist for a more objective picture of driving ability than family observation alone. NIA directs families to such specialists, a health professional or their state DMV to find an evaluator, and notes that assessment fees may apply.

When you contact a program, ask whether its service includes a formal on-road component, what happens before that component, and whether your parent's medical and license status permit it. Some state licensing processes also use road testing; California's is one example below. An occupational therapist doing a home-safety visit is not automatically the driving specialist you need. Describe the service you're seeking rather than relying on a job title.

Ask about fees, referral requirements, waiting lists and who receives the report. Confirm coverage with the provider and insurer. When driving is unsafe or restricted, arrange another driver to the evaluation. The appointment doesn't authorize the journey behind the wheel.

If the clinician advises stopping, don't wait for a road test to follow that advice. The Association suggests asking the physician to explain the restriction and put it in writing. Clarify the next step and follow-up. If driving is permitted, obtain explicit limits and a reassessment plan consistent with state requirements.

I distrust the family bargain that says "only to the grocery store" without an individual assessment. Mama's difficulty was on that very errand. Don't invent a daytime, short-distance exception to a stop-driving recommendation because it sounds easier to enforce.

Say what happened, then say what changes

NIA's older-driver guidance suggests preparing alternatives before the talk, using "I" statements rather than accusations, and keeping the conversation about safety and continued access to activities. The Association recommends acknowledging the pain of giving up driving while remaining patient and firm.

Use Chapter 12's communication adjustments away from an imminent departure. Keep your parent involved. Bring an actual offer of help, not a promise that the family will somehow handle everything.

Here is a script to adapt when you've observed a concern and are arranging assessment:

"Dad, I'm worried about what happened at the intersection. I want us to ask Dr. Lee about driving. I'm taking you to the appointment, and I'll drive for now while we get advice. We can still go to the hardware store. Let's choose when."

Dr. Lee is a hypothetical name. Use your own observed event and help you can provide. With only a neighbor's report, say so.

When the clinician has advised stopping, the sentence changes:

"The doctor has said you must not drive. I'm driving today. I know this is upsetting. We can talk about where you want to go."

Don't claim a doctor's order you haven't received. Don't say "just for a week" if nobody has promised a review then. Firmness doesn't require an invented deadline or an invented authority.

Your parent may disagree, and you may have to return to the conversation. The Association cautions that impaired insight and judgment can make the risk hard for the person to recognize. An angry response isn't evidence that the advice is wrong; a calm response isn't proof the arrangement will be followed. Check access, not just agreement.

Ask the clinician to join a family conversation if needed. Tell other relatives what was recommended and what ride help is needed. If a

sibling disputes the restriction, have them raise the actual medical question with the care team rather than quietly lend another car. Don't build a family courtroom around your parent.

Your state has its own reporting route

NIA's dementia-driving page states that laws vary by state, with some requiring physicians to report a dementia diagnosis and different licensing review and testing procedures. A clinician's safety recommendation, a specialist's evaluation and the state's licensing decision are related, but they aren't interchangeable.

Find the agency where your parent holds the license through [USAGov's state motor vehicle services directory](#). On the official state site, look for medical review, dementia and reporting an unsafe driver. If the instructions aren't clear, call its medical-review or driver-safety office. A search result about another state is not your parent's rule.

Ask how a family member can report a specific concern, what medical documentation is requested, and whether the clinician has a mandatory-reporting obligation. Ask the clinician about their own obligation too. Find out what the driver must do next, what restrictions apply during review, and how notices or review rights work. Don't assume a report is anonymous or that it immediately cancels a license.

For a verified example, the [California DMV's dementia page](#), checked in October 2026, says that when DMV receives a report about a driver with dementia, it follows up with a driver medical evaluation. It describes reexamination and possible supplemental driving testing for mild dementia, and revocation for moderate or severe dementia. Those are California's published procedures, not a national stage-based licensing rule. Check the current procedure in your parent's state rather than assigning a stage yourself.

Keep state directions and notices together. “The doctor’s handled the DMV” isn’t confirmation. Check who is submitting what; today’s safety measures cannot wait for paperwork.

If authority, vehicle ownership or the reporting process is contested, consult a licensed attorney familiar with your state’s elder-law and licensing issues. Being the adult child is not a substitute for the required authority to sell somebody else’s car. Chapter 16 owns the broader paperwork questions.

When the keys are still being used

The Association and NIA describe removing keys or disabling a car when someone continues driving unsafely; the Association calls these last-resort measures to pair with reliable alternative transportation. They address unresolved danger, not discomfort with an early conversation.

If your parent continues driving against advice, secure every accessible key and fob, including spare sets, as soon as you can do so safely. Tell the people helping with care not to return them or lend a vehicle. A fob in a visitor’s coat can undo the arrangement just as easily as a spare in a drawer.

Keys may not be enough. If vehicle access remains possible, ask a qualified mechanic or vehicle professional how to prevent use safely. Don’t disconnect parts yourself or install a hidden switch based on this book. Any professional disabling arrangement needs to prevent use, not leave a partly working vehicle that can be driven into trouble. Vehicle relocation or sale also raises permission and ownership questions; get appropriate help rather than signing for your parent without authority.

Never turn this into wrestling over keys. Don’t stand in the vehicle’s path, reach into a moving car or grab the steering wheel. If you cannot prevent an imminent unsafe departure safely, or your

parent is driving and presents an immediate danger, call 911. Give the vehicle description, known location and direction, the dementia diagnosis and the reason for concern. Follow the dispatcher; don't chase the car yourself. An emergency call isn't the same as filing a routine DMV report.

If the person and vehicle cannot be located, use Chapter 13's missing-person action card without delay. A tracker is not a substitute for that call.

A secure-access-now card

Use this when driving continues against advice, not as a substitute for assessment or state review.

1. **Immediate danger:** Call 911 if an unsafe departure cannot be prevented safely or dangerous driving is underway. Do not wrestle, block or chase.
2. **Key access:** Safely gather every key and fob, spares included. Inform helpers: no key handbacks or borrowed cars.
3. **Vehicle access:** If needed, obtain qualified help to prevent vehicle use safely. Resolve ownership or authority questions with legal advice; no improvised disabling.
4. **Professional follow-up:** Promptly ask the care team about today's risk; follow your licensing agency's current reporting process. A future appointment isn't today's safeguard.
5. **The next trip:** Name the driver or service, pickup arrangement and needed companion. Confirm the return trip and a backup.

Keep operational details available to agreed helpers, not on public display. Chapter 13's usable-exit rule still stands: preventing car use is not permission to confine your parent at home.

Replace the trip, not just the driver

“Call me if you need anything” leaves your parent arranging every outing. Book a trip with someone who has agreed to drive. Include an ordinary pleasure. The examination room shouldn’t become the only destination.

NIA names family rides, community transport and local volunteer possibilities. The Administration for Community Living’s [Eldercare Locator](#) can connect you to local aging services; ask about transportation in your parent’s area. The Association also suggests taxis and deliveries of groceries, meals or prescriptions. Availability, costs and eligibility need local confirmation. None of those options automatically supplies the supervision your parent needs.

Ask a proposed service where the driver’s help begins and ends. Can somebody meet your parent at the door? Who stays during the appointment, and who helps them get home? Tell the service the assistance required and confirm whether it can provide it. A booked car isn’t an adequate arrangement if your father cannot recognize it, explain his destination or wait safely alone.

If another adult must accompany your parent, name that person separately from the driver and confirm both sides of the trip rather than leaving “return ride” blank because getting there took all your attention.

Delivery removes an errand, not the pleasure of going out. If the store visit mattered, arrange an accompanied outing when feasible. Let your parent choose between workable times even when driving is no longer an option.

Write the next destination your parent wants to reach and the person who has agreed to drive. Confirm the trip today. Give your parent something to put on their coat for.

Chapter 15: Sundowning and Difficult Behaviors

The lamp beside Mama's chair was still off. It was near four in the afternoon, and Dot had begun getting up, crossing the room, sitting, then getting up again. I was watching the clock because I had noticed that hour before.

Toni, a friend from my church caregiver group, suggested turning on the lamps earlier, offering a snack and taking a walk together if Mama could do it safely. Toni had been caring for her husband longer than I had been helping Mama. She had ideas, not a prescription.

In this composite story, I logged times for a month before testing those suggestions against the afternoon pattern. Meanwhile, I responded to Mama's needs and brought concerns to her clinician. Comfort and medical help didn't wait for the notebook.

I wanted to change everything at once. Earlier lamps. Different supper. No visitors. Then, if an evening went better, I would have no idea which change mattered, or whether any of them did.

I'll tell you what helped: putting the time beside what I actually saw. *Agitated* was a word. Getting up repeatedly near four gave me something to discuss.

Return to this chapter as behavior or sleep needs change. A new or dangerous change needs help now, not a completed log.

The hour is a clue, not an explanation

The Alzheimer's Association's [Sleep Issues and Sundowning](#), checked in October 2026, describes increased confusion from dusk through the night. Anxiety, agitation, pacing, disorientation and difficulty sleeping can be part of it. Some people experience hallucinations. The Association calls sundowning a set of symptoms, not a separate disease, and says its exact cause is unknown.

The word describes a pattern you and the clinician may recognize; it doesn't establish that an unfamiliar behavior is harmless because it happened after supper.

The Association names possible contributors: exhaustion after the day's activities, changes in the internal body clock, and low light that creates confusing shadows. A new or confusing environment can add difficulty. Stress or frustration in people nearby may add to the person's distress.

Notice *may*. A crowded afternoon might precede pacing, while another afternoon with visitors goes comfortably. Bring the difference to the care team rather than deciding that company has become impossible.

A mixed-up body clock means the person may be tired during the day and awake at night, according to the Association. Don't force an exhausted parent to stay awake. Ask the clinician whether changing late naps is appropriate for this person's sleep needs.

Caregiver stress on that list doesn't mean you caused the problem. I wouldn't accept that interpretation. Use it to ask for a quieter evening or another person's help, not to blame your temperament.

"What happened before the pacing?" gives you something to observe without asking you to solve a problem the source itself says is not fully understood.

A new change goes to the clinician first

An abrupt change, a new symptom or a much more severe episode needs medical help, not the familiar explanation. Don't wait for a repeat.

The National Institute on Aging's [Managing Personality and Behavior Changes](#) says sudden or rapidly fluctuating behavior changes should be brought immediately to a doctor's attention, especially after an infection or recent medication changes. Its possible contributors include pain, constipation, hunger, thirst, sleep loss and medicine side effects. The Alzheimer's Association's [Anxiety and Agitation](#) also names medication interactions and calls for medical assessment of behavioral symptoms, especially when they appear suddenly.

Use Chapter 10's emergency boundary: sudden confusion or possible stroke signs mean 911, even at the usual difficult hour. MedlinePlus's [Confusion guidance, reviewed in 2026](#), gives the sudden-confusion emergency instruction. Note onset without delaying help. Don't try lamps or a walk first.

If you or your parent is in immediate danger, call 911 and follow the dispatcher. Get to a safe place if you can. You do not need to stand close enough to be hit while searching for a calming sentence. The National Library of Medicine's [dementia behavior and sleep guide, reviewed in April 2026](#), puts danger ahead of a home strategy.

For a sudden behavior change without those emergency features, contact the treating clinician immediately for assessment instructions; use the office's urgent or after-hours route rather than waiting for the next visit. Say what changed and when. If you cannot reach the team, seek urgent medical help rather than substituting the log for advice.

Tell the clinician about pain your parent reports, reduced drinking, changes in urination, illness and recent medicine changes. Ask whether infection, dehydration or a medication problem could

be contributing. These are possibilities to assess, not a home diagnosis. Confusion alone doesn't tell you that a urinary tract infection is the cause, and the book isn't a reason to start antibiotics or change medicines.

The Association's sundowning page says urinary tract infections, restless legs and sleep apnea can cause or worsen sleep problems. Report persistent sleep disruption too: whether both of you can sleep and whether care is still safe, not merely whether your parent is quiet.

Watch what comes before, without waiting to help

Once medical concerns and immediate safety are addressed, begin a short record while following the clinician's instructions. My month of notes isn't a required waiting period.

The Association recommends recording what happens before sundowning episodes to look for possible triggers. NIA also says tracking behavior changes can help caregivers and health professionals recognize patterns. Neither instruction makes you responsible for proving the cause.

Write an observable event instead of a character judgment. "Walked between the chair and hallway; asked where supper was" gives somebody else a picture. "Impossible again" records how hard the evening felt, but not what your parent did. You can note your own distress separately. It matters too.

Note what preceded the episode: an appointment, television playing under several conversations, or a rest. No guessing. Pacing after an appointment isn't proof that medical visits should stop.

Include an observed evening without the problem for comparison, so the page isn't a collection of only the worst moments.

Record what you offered and what followed. Sitting down after a lighting change doesn't establish that darkness caused the pacing. No change? Write that too.

Don't interrupt needed comfort to preserve a neat comparison. You can help with several needs at once and record that you did. For a nonurgent environmental change the care team agrees is suitable, choosing one adjustment at a time can make your notes easier to interpret. That is a practical way to organize observations, not a clinical experiment or a demand for another bad night.

If the clinician asks for a particular record, use theirs. Take the relevant entries through the Chapter 11 note route; you don't need to recreate its health summary. Ask what would justify an earlier call while the pattern is being reviewed.

Give the evening less work to do

Start with a change that fits something you observed. You don't have to carry out every suggestion on a website before asking the doctor for help.

The Association recommends regular waking, meal and bedtime routines, demanding activities earlier in the day when the person is more alert, and less evening stimulation. NIA's [Managing Sleep Problems in Alzheimer's Disease](#) offers similar timing advice. If an afternoon appointment leaves your parent exhausted, ask whether the next one can be earlier. Chapter 15a handles the daily-care tasks themselves.

Earlier lighting was Toni's suggestion because I had described an afternoon with the lamps still off. The Association recommends keeping the home well lit in the evening to reduce confusion. Aim for useful light, not glare. Its agitation guidance names glare as a possible trigger; look from the chair your parent uses instead of judging the room from the doorway.

At bedtime, NIA recommends a restful setting and nightlights along needed routes. Lighting for an awake person during a difficult evening and lighting for sleep are different questions. Use Chapter 13's safe walking routes, and ask the care team how to balance visibility with rest. Don't create darkness someone has to cross to reach the bathroom.

A quieter evening could mean turning off the news or saving the vacuuming for another time. The Association suggests soothing activities the person enjoys, such as familiar music or photographs. A favorite film may suit one parent; a screen may upset another. Observe the response rather than completing an activity because it was on a list.

Daylight and an accompanied walk are also suggestions on the sundowning page. Ask what activity is appropriate for your parent's health and mobility. Choose a safe place and suitable conditions, not a prescribed distance. A walk isn't a treatment for an abrupt medical change, and exercise is not an obligation to exhaust somebody into sleeping.

The page suggests making lunch larger and supper lighter. Discuss food changes with the care team if intake, swallowing or a prescribed eating plan is a concern. A lighter evening meal doesn't mean withholding food from someone who needs it. Toni's snack suggestion was an offer to consider, not a rule that agitation means hunger.

If your parent paces, the Association advises supervision rather than restraint. Keep needed help nearby and the Chapter 13 safety arrangements in place. Don't hold the person down, trap them behind a door or tie them into a chair to obtain a quiet evening. When safety cannot be maintained, get professional or emergency help as appropriate; don't improvise a restraint.

Medicine decisions need a separate conversation. The Association recommends asking the doctor about medication timing and

discussing risks and benefits if treatment is proposed. Don't move doses yourself. MedlinePlus's behavior and sleep guide says to consult the provider before giving store-bought sleep medicines because some can worsen confusion. Keep the Chapter 11 medicine boundary, including nonprescription products and supplements.

If an agreed adjustment doesn't help, report back rather than adding remedies. Ask what comes next and when to review it.

An accusation is not a verdict on your love

"You took my purse" can hurt differently from pacing. You may have spent the whole day helping and find yourself named as the person causing harm. You are allowed to feel wounded without making your parent settle that feeling for you.

The Alzheimer's Association's [Suspicious and Delusions](#) explains that confusion and memory loss can contribute to false beliefs, including theft accusations. A delusion is a firmly held false belief; a suspicious one may be described as paranoia. A hallucination is different: seeing, hearing or otherwise sensing something that isn't there. Tell the clinician what happened rather than using either word as your own diagnosis.

Don't assume every accusation is false because your parent has dementia. Money can actually have been taken. NIA's [Elder Abuse guidance](#) says to report suspected mistreatment; you needn't prove it yourself. Contact local Adult Protective Services for help with suspected abuse at home, or 911 for immediate danger. Don't confront a suspected wrongdoer in a dangerous situation.

For a recurring suspicion the care team has assessed, the Association recommends listening to the worry and not taking offense. Chapter 12's brief-answer approach applies here; there is no need to deliver a defense of your entire character.

You might say, “Your purse matters to you. I’ll help look.” Say that only if you can help look. Don’t agree that somebody stole it, invent a culprit or promise to recover something you cannot locate.

Acknowledging distress does not require endorsing an accusation.

The Association suggests duplicates for an item repeatedly thought lost. That might mean a spare ordinary comb. It should not mean creating duplicate bank cards or giving back unsafe car-key access. Choose a safe item and discuss persistent searches with the care team. A replacement isn’t permission to ignore a real loss.

If suspicion centers on another helper, report the actual words and circumstances privately to the appropriate care professional. Don’t circulate a humiliating account among relatives or let an unverified accusation become a family verdict. You need help understanding it, and the helper may need a different arrangement while the concern is assessed.

Severe or very troubling delusions or hallucinations require medical evaluation, according to the Association. Report fear, lost sleep or unsafe behavior before someone is injured. This needs more than a better sentence.

A behavior log you can actually keep

Use a notebook or the format your clinician requests. Date each entry and identify who observed it. Keep it somewhere agreed caregivers can use appropriately, not on public display. This page is for a pattern, not a score of how well either of you behaved.

Help comes before logging: 911 for sudden confusion, possible stroke signs or immediate danger. Contact the clinician immediately for other sudden behavior changes.

- **Time and observation:** When it started, what your parent did or said, and how long you observed it. Leave an unknown start time unknown.

- **What came before:** Activity, rest, visitors, lighting or noise you actually noticed. Mark another observer's account as theirs.
- **Comfort or health concern:** What your parent reported and what you saw, including pain or reduced drinking. No guessed diagnosis.
- **Recent change:** A known illness, care change or clinician-directed medicine change; include dates if known.
- **What you offered:** The specific adjustment or help, whether accepted, and whether several things changed together.
- **What followed:** The observable response, no change, or something worse. Record uncertainty without assigning a cause.
- **Help and follow-up:** Who was contacted, instructions received, and the agreed next review. Emergency help is never deferred to fill this field.

For an invented entry, write: "Late afternoon; walked between chair and hallway while TV news played. Turned television off; stayed nearby. Sat down afterward. Don't know whether quieter room helped." No diagnosis has been made. You have described something the clinician can discuss.

Keep the blank spaces that matter. If you weren't there overnight, don't fill in "slept well" because the morning seemed peaceful. If your parent declined a suggested activity, write that instead of marking it a failure. Their response belongs in the account.

You can put the pen down when the entry is done. Your parent doesn't need to spend the rest of the evening being watched for the next sentence. Before the next familiar difficult hour, leave a fresh dated page beside the lamp switch. If the behavior is new, make the call first.

Chapter 15a: Daily Care — Bathing, Dressing, and Eating

The daughter has the shower running. Her mother has both hands around the edges of her cardigan and isn't taking it off.

In this imagined bathroom, the daughter says it's time to wash. Her mother says no. The daughter reaches for the sleeve, then stops. She turns off the water and holds up the towels already waiting on the counter: cream or green.

Her mother points to green. The daughter puts it within reach. They sit for a moment before her mother agrees to a wash.

The towel gave her one small decision and stopped the pulling. Tomorrow may call for something else.

This month's job is a workable daily-care routine, not a timetable that makes you both late by breakfast. Start with the task needing help now. A sudden loss of ability needs medical attention first.

Keep the familiar order, leave some space

The Alzheimer's Association's [Daily Care Plan](#), checked in October 2026, recommends considering the person's old habits, preferences, abilities and best time of day. It asks caregivers to allow ample time for meals and personal care, with flexibility and a balance of activity and rest.

If your father has always eaten before getting dressed, begin there unless his care instructions require otherwise. A printed schedule that puts washing first isn't a reason to reverse his preference.

“After breakfast, when she’s ready” can be a useful place for dressing, not a deadline that permits rushing someone who is still chewing. Keep prescribed medicine, meal or other medical timing as directed. Flexibility isn’t permission to move doses.

A routine saves inventing the whole morning repeatedly, though the help needed can change. Lay out supplies, see what your parent can do, and help with the difficult part. One sleeve needing assistance doesn’t require taking over.

Leave an empty stretch after a demanding task. The Association specifically cautions against filling every minute. Rest doesn’t need to earn its place by producing a completed activity afterward.

I would rather have a plan another helper can understand than an impressive schedule nobody can carry out. “Clean clothes in the basket beside the dresser” is usable. “Maintain independence” doesn’t tell the next person where the socks are.

Chapter 15 owns the evening behavior pattern; here, you’re arranging personal care with the help actually available.

Get the towel ready before asking for the bath

According to the Association’s [Bathing guidance](#), receiving intimate help can feel embarrassing or threatening. Cold, discomfort and difficulty judging depth can also make bathing frightening. Resistance isn’t proof that your parent has decided to make your morning hard.

Prepare before inviting them into the room. Have the towels, washcloth, chosen cleanser and clean clothing ready. Check that the room is comfortably warm and test the water yourself before it touches your parent. The Association warns that the person may not recognize dangerously hot water, even when they have run the bath themselves.

No dangerous heater beside the shower. Chapter 13's professional bathroom-safety assessment still applies.

A shower chair or bath bench may make seated washing easier, according to the Association. Ask an occupational therapist which equipment fits your parent and the shower, and have it fitted properly. No kitchen chair substitutes. If getting in, sitting down or standing up is uncertain, obtain an assessment and hands-on instruction before attempting the transfer, rather than lifting your parent into the tub yourself.

Explain what you're about to do before removing clothing or touching an intimate area. Close the room to unnecessary observers without trapping anyone inside. Keep a large towel around the person where practical, uncovering only what needs washing. Privacy isn't less important because you are family.

Offer a real choice: cream towel or green, a wash now or another suitable time. Use Chapter 12's one-step approach. Holding the washcloth or washing their own arm gives your parent a part; neatness doesn't matter.

If the person refuses or becomes frightened, stop and settle them safely. The Association suggests trying later and considering alternatives. The towel choice isn't permission to disregard a continuing no, pull off clothing or restrain your parent.

Full showers aren't the only way to provide hygiene. The Association suggests washcloth washing between showers and separating hair washing from the rest of the bath. Ask the care team what cleaning is needed for this person's skin, mobility and continence needs. No universal shower frequency belongs on your planner.

Essential cleaning after an accident still needs to happen, but do not force a shower or attempt an unsafe transfer. If a washcloth clean-up is not safely possible, call the clinician's office, home-health agency, or local caregiving resource the same day for safe-care

instructions and help; use urgent or emergency help if the situation is medically urgent or you cannot keep either person safe.

Dry gently, including skin folds, and help with dressing from a safe seated position if that is the assessed arrangement. The Association recommends watching for rashes and sores. Report those to the clinician rather than treating a new skin problem with whatever powder is in the cabinet.

One outfit, not a cupboard full of decisions

The Association's [Dressing and Grooming](#) recommends simplifying choices, laying clothes out in the order they go on, and handing over one item at a time. Give your parent a choice between two suitable outfits if that helps, then put away the unchosen one.

Today's clothing only. A shirt lying across the undergarments can hide the first step; lay the pieces where your parent can see them rather than expecting a spoken list to remain available throughout dressing.

Comfort and ease matter more than the outfit's elegance. The Association suggests soft, loose-fitting clothes and simpler closures in place of difficult buttons or zippers, plus comfortable nonslip shoes. Elastic-waist trousers may remove a difficult fastening at bathroom time. Ask the occupational therapist about clothing adaptations if movement, balance or hand use makes dressing hard.

Watch the actual problem. A pullover isn't automatically easier: a parent who cannot comfortably raise an arm may need a different garment and medical assessment, not another instruction to try harder.

Mismatched colors are fine. Clean and suitable for the weather is enough. The Association suggests similar or duplicate clothes when someone wants the same outfit repeatedly, not a project to reorganize the house.

Grooming can keep an old preference alive. The Association recommends familiar toiletries and continuing barber or salon routines where feasible; if going out becomes distressing, ask whether a home visit is available. Show the comb being used rather than leaving several tools in front of your parent. Keep sharp grooming tools out of unsafe use and seek qualified help for tasks you cannot do safely.

Teeth aren't optional. The Association's [Dental Care guidance](#) recommends twice-daily cleaning and adapting to the person's comfort. Show the toothbrush and give accepted help. When brushing or denture care becomes difficult, ask the dentist for a home-care plan rather than forcing a mouth open.

Wincing over a meal or refusing food may signal mouth pain or poorly fitting dentures, according to that guidance. Contact the dentist about those signs; tell the medical team about reduced intake too. If rinsing or swallowing is difficult, ask for individual instructions rather than automatically offering mouthwash or water to someone who cannot use it safely.

Make the meal easier to notice

A plate left beside the chair isn't proof that lunch was eaten. The Association's [Food and Eating](#) explains that a person may not recognize food, may forget to eat, or may become overwhelmed by choices. Medicines and painful dentures are among the possible contributors to poor appetite.

Clear away the unopened post. The Association recommends fewer distractions, a simple table setting, only the utensils needed, and enough contrast to distinguish food from the plate. A plain plate on a contrasting placemat may be worth trying.

Serve a manageable amount, with one food at a time if a mixed plate is confusing. Check food and drink temperatures before

serving. Allow time. Sitting with your parent and eating your own meal can give the occasion its familiar shape without a running commentary about every bite.

If handling utensils is the obstacle, a bowl, easier-to-hold utensil or food picked up by hand may help. Manners can wait. Choose food consistent with existing eating and swallowing instructions; the name “finger food” doesn’t establish safety.

Offer another suitable food rather than proving yesterday’s dish used to be their favorite. Never force-feed. Persistent refusal, weight loss or a clear drop in intake calls for the treating clinician, with a dietitian if recommended. Supplements and prescribed diet changes belong with that team.

Offer small drinks through the day rather than expecting your parent to remember the jug. The Association recommends frequent small cups and foods with high water content. Use drinks and foods allowed by their care plan. If your parent has a prescribed fluid limit, ask the clinician how these offers fit within it; this book doesn’t supply a daily target or override the restriction.

Report reduced drinking promptly, especially when your parent seems unwell. An untouched cup doesn’t establish lack of thirst. Chapter 10’s sudden-confusion emergency boundary applies before offering a drink.

Swallowing trouble needs a different kind of help

The Association warns that swallowing problems can lead to choking and weight loss. The American Speech-Language-Hearing Association’s [adult swallowing guidance](#) names coughing during meals, a wet or gurgling voice, and food remaining in the mouth among signs requiring attention. It explains that a speech-language pathologist can assess swallowing and recommend an individual plan.

If you notice swallowing difficulty, stop further bites or drinks during the episode and obtain prompt clinical advice. No water to wash food down or improvised thickened-drink recipe. Ask what intake is safe while awaiting assessment. Seek urgent medical care if timely advice and safe intake aren't possible.

Food texture, drink thickness, cups, straws and positioning need individual instructions when swallowing is a concern. A softer food isn't automatically a safe food for every swallowing problem. Follow the professional's plan and ask them to demonstrate the help required.

Severe choking is an emergency, not an appetite problem. MedlinePlus's [choking first-aid guidance, reviewed in February 2026](#), distinguishes a strong cough from inability to speak or breathe adequately. Call 911 immediately if your parent cannot breathe, speak or cough effectively, or becomes unresponsive. Follow dispatcher instructions and use your current first-aid training. Arrange practical choking-response training before you need it; this paragraph doesn't teach the procedure.

Wet clothing is a care task, not a character flaw

Incontinence means loss of bladder or bowel control. The Association's [Incontinence guidance](#) describes it as common as Alzheimer's progresses, with several possible causes and ways to manage it. It isn't a prediction that your parent will have it at a particular stage, and it isn't a reason to scold.

New leakage needs medical assessment. The Association lists infection, constipation, medicine effects and difficulty reaching or recognizing the bathroom among possible contributors. Ask the clinician to assess the change rather than deciding it is dementia progression. Keep Chapter 11's medicine boundary; don't omit a prescribed medicine because the laundry has increased.

Once causes and care needs have been assessed, offer bathroom help at familiar points in the day, such as on waking, after meals and before bed, adjusted to your parent's observed needs. The Association recommends regular reminders and allowing enough time. There is no compulsory interval here and no reason to insist someone sit until the schedule is satisfied.

A clear bathroom cue and easy-to-remove clothing may help. Use Chapter 13's reviewed bathroom arrangement, including the needed assistance. Ask an occupational therapist whether a toilet aid or bedside commode is appropriate and how to use it safely; don't buy equipment without considering the transfer and who will help.

Absorbent pads, protective underwear or adult briefs are options the Association names, along with waterproof mattress protection. Ask a nurse or continence professional about fit, changing and skin care. Products should support timely cleaning and comfort, not replace bathroom offers or needed assessment.

Keep supplies privately stored nearby. Wash and dry the affected skin gently as instructed, and report redness, sores or pain. "Let's get you comfortable" is enough; the whole house doesn't need an announcement.

Restricting drinks to save bedding isn't a care plan. The Association warns against causing dehydration by limiting fluids. Follow any clinician-prescribed fluid restriction, and ask about timing rather than cutting drinks yourself.

The routine planner beside the clean towels

Use this dated page for an ordinary task, not Chapter 15's behavior log. Follow Chapter 11's agreed private handoff. Start with the hardest task; add another only if useful.

Call 911 for sudden confusion, possible stroke signs, severe choking or immediate danger. For other abrupt

losses of ability, contact the treating clinician immediately, using urgent or after-hours help if needed. Paperwork waits.

- **Task:** Name the actual job: washing, putting on trousers, lunch, a drink offer or bathroom help. Include what your parent still does themselves.
- **Time of day:** Write the familiar part of the day or suitable window, not a compulsory deadline. Preserve prescribed medical timing.
- **Simplification tried:** One outfit laid out, supplies ready, separate hair washing, fewer utensils, or another suitable adjustment. Record only what you actually tried.
- **What helped:** The observable response, what was declined, and what remains uncertain. A finished task doesn't prove the method caused success.
- **Help required:** Name the agreed helper, needed supervision and any professional instructions or equipment assessment still pending.
- **Escalation and follow-up:** Keep the clinician's urgent contact and individualized reporting instructions here. Record the professional contacted, advice received and who follows up; emergencies come first.

A sudden new inability to dress, eat or tolerate washing needs the doctor before the occupational therapist. The National Institute on Aging's behavior-change guidance in Chapter 15 identifies pain, infection and medicine changes among matters needing assessment. Occupational therapy can then address the task and equipment, not replace medical evaluation.

For a nonurgent, continuing difficulty, ask the clinician about an occupational-therapy referral and describe the troublesome step. Say

“needs help getting from standing to the shower seat,” not “needs better bathing.” Ask for a demonstration and confirmation that the available helper can provide the assistance safely.

Write one task on the page today. If it is washing, put the chosen clean towel where your parent can reach it. Leave the unchosen towel folded.

Chapter 16: Legal and Financial Paperwork To Do Now

The teller was polite about it. She could not discuss the account, and she could not say whether the electric bill had gone through.

In this imagined branch, the daughter has her mother in the car outside and five months of groceries, pharmacy runs and opened mail behind her. What she does not have is a piece of paper saying she may ask about her mother's checking account.

So she drives home knowing nothing, and the electric bill stays a mystery until the next statement.

That gap is the whole chapter. Daily care often goes to whoever shows up. Access to money or medical information, and authority to make decisions, depend on the institution, state law, and a valid document, authorization, or court order.

Some situations need prompt advice from a licensed elder law attorney, especially when capacity is uncertain, documents conflict, or family members disagree. Ask the attorney or your state health agency what your parent can complete without one.

This month's work is narrower than it sounds. Find out what your parent already signed, find out what is missing, and find out who in your state can tell you whether your parent can still sign now. You are not becoming an expert on any of it.

Why the calendar matters more than the filing cabinet

The Alzheimer's Association's [Legal Documents](#) page, checked in October 2026, says a person living with dementia keeps the right to make their own decisions for as long as they have legal capacity. It also says plainly that these laws differ from one state to another, and it recommends a signed will be put in place as soon as possible, while the person is still able to decide.

Read that twice, because both halves matter. Your parent's diagnosis did not remove their say. And the window for them to have a say is not permanent.

What counts as legal capacity for a particular document, who judges it, and what happens if somebody later disputes it are questions for a lawyer licensed where your parent lives. Not for me, not for a website, and not for a well-meaning cousin. Chapter 10's stage labels cannot answer it either; a label is a general description, not a legal finding about your mother and one document on one afternoon.

So the honest version of the advice is this. Talk to an elder law attorney soon if you are unsure whether your parent can still sign the documents your family needs. Ask how capacity gets assessed in your state.

The National Institute on Aging's [Getting Your Affairs in Order checklist](#), reviewed in 2023 and still posted when I checked it in October 2026, says to ask about a lawyer's fees before making the appointment. A professional office should be able to explain its fee structure before you book; if the answer is unclear, compare another office.

I'll tell you what I would not do: wait until the family agrees. Agreement is lovely and it is not a prerequisite for asking a lawyer what your state requires.

The documents to ask about, and nothing more

This book names the documents rather than teaching them.

A power of attorney for finances lets your parent name someone to handle money matters. A power of attorney for health care names a person to make medical decisions when your parent cannot; the Association's page notes this kind of document is also called an advance directive. A living will is a different advance directive, covering treatment wishes. MedlinePlus, updated in January 2026, describes [advance directives](#) as legal documents that let someone set out end-of-life care decisions ahead of time, with the living will covering treatments and the durable power of attorney for health care naming a proxy.

Two practical notes that are mine, not any source's.

First, bring your parent into it while they can tell you what they want, not merely sign what somebody drafted. The Association's [Planning for Care Costs](#) page makes the same point about money talks happening early, when the person can still understand the issues and say what they wish. A document records a decision. Someone has to make the decision first.

It also recommends identifying which needed documents are not in place, with professional help if required.

Second, resist the informal shortcuts that look easier this week. Adding your name to an account, keeping your father's debit card in your wallet, using your mother's online banking password: each of those is a question for the attorney, not a project for Saturday. Chapter 11 already said not to borrow a login for medical records. The same instinct applies to money, with an extra reason. If anyone ever asks where the money went, you want paperwork and not an explanation.

There is also a court route when no valid document exists and decisions cannot wait. The Association calls it guardianship or conservatorship, says it takes time and an attorney, and says the

rules differ by state. If decisions cannot wait and authority is unclear, talk to an elder law attorney familiar with your state's rules.

Never coach the signature

A pen is not a plan.

If your parent does not understand what they are signing, a signature creates a problem rather than solving one, and the problem surfaces years later when somebody needs the document to work. Don't hand over a page and guide the hand. Don't present the same request fifteen times until you get a yes on the sixteenth. Don't let the sibling with the strongest opinion run the meeting.

If anyone in the family questions whether your parent understood, stop. Put the pen down, get legal advice before going further, and say so out loud to everybody involved. An attorney can tell you what your parent's state requires and how to document it. You cannot settle that question by being certain.

The same goes for a signing that happens on a bad afternoon. Ask the attorney how to arrange it instead of pushing through. Offices that do this work regularly have seen the problem before.

That's fear, by the way, when it rises in your chest at the word *capacity*. Set it down long enough to make the phone call. Guilt and fear are not planning tools, and neither one has ever filled out a form.

For the document choices, state forms, and how to prepare for a lawyer, use *Estate Planning for Beginners*; if capacity or authority is uncertain, contact a licensed elder law attorney promptly.

A folder the next caregiver can find

Locate, without changing anything: existing estate documents, the original's location, current account/bill contacts, insurance

information, and the name of the person or lawyer who may know more. Bring that list to the attorney. Do not sign, transfer, close, or add anyone to an account based on this list.

Start with your parent's agreement to look together. Ask where the papers are, rather than asking them to explain every account from memory. "Can we look in the drawer where you keep the insurance letters?" gives you somewhere to begin. Use Chapter 12's shorter approach; this isn't a test of what they remember. If your parent objects or permission is unclear, pause and ask the appropriate professional about access instead of searching secretly.

You can do a little locating without making a decision. Finding the insurance envelope doesn't require discussing a change of policy. Finding the deed doesn't require discussing a house sale. Leave those decisions alone. If the conversation becomes tiring, note where you stopped and put the papers back where your parent expects them.

For the caregiver's location note, write "signed papers, blue folder, bottom desk drawer" rather than "documents at home." Add the date you actually looked. If your parent says the lawyer has the original but you haven't confirmed that, write "parent recalls lawyer has original; not confirmed." A recollection is useful, provided it doesn't quietly become a fact.

Keep different versions together without choosing which controls. A later date or a familiar signature isn't your verdict on validity. Don't discard an older document, fill a blank, or remove pages. Flag the uncertainty for the attorney. Your job here is to preserve what you found, not repair it.

NIA's checklist recommends keeping important papers or a record of their locations in one place and telling a trusted person where to find them. It also advises home copies when originals are in a bank safe deposit box. Ask the bank about authorized access rather than assuming that finding a key allows you to enter the box. Don't postpone the professional call until you have searched every drawer.

Keep the location note private. A relative bringing supper doesn't need account details. With the appropriate permission, tell the agreed backup caregiver where the needed copies and contacts are kept; confirm that they can find them. Don't put account numbers, signatures or identity documents into the family group chat. If you make an authorized digital copy, use protected storage and share only with the intended recipient through a route they confirm.

No paperwork outranks a medical emergency; Chapter 10's rule about calling 911 stands.

Permission is its own piece of paper

Back to that bank counter.

NIA's checklist makes a distinction worth carrying into every office you deal with. Giving permission in advance for a doctor, a lawyer, an insurer, a credit card company or a bank to talk with your caregiver is a separate act from naming a health care proxy, and it may mean signing and returning that organization's form. A proxy only steps in when your parent cannot communicate their own decisions.

So ask each institution what it needs, in its own words, and ask while your parent can still complete it. The bank's form, the insurer's form, the doctor's office arrangement from Chapter 11: all different, all theirs. I don't promise any of them will accept what you bring. Ask what they will accept, and write down who told you.

Describe the immediate task precisely. "I need to ask about a bill" is different from "I need to move money." Permission to discuss information isn't permission to make a transaction. Ask the institution which of those tasks its proposed arrangement covers; don't infer the answer from being allowed onto a phone call.

A form mailed back isn't a finished job in your caregiver notes. Record when you sent it through the institution's approved route,

then confirm receipt and ask whether anything remains missing. If it isn't recorded yet, keep the task open. An envelope on its way and an authorization the office can find are different things.

Save the office's answer with its contact details, away from ordinary family updates. Ask how your identity will be verified on the next call and how a permitted helper should contact them. Follow that process rather than answering as your parent or borrowing their password. If your parent must participate, arrange a suitable time with the office; don't rehearse answers or supply them on your parent's behalf.

If access is refused, note what the staff member says is missing. "They need to review the document" is more useful than "the bank won't help." Ask for the institution's review contact. A disputed document, uncertain capacity or unresolved authority goes back to the licensed attorney, not to another attempt with a different teller. Where a bill or care decision is time-sensitive, say what the immediate problem is when seeking help; collecting papers is not a reason to wait.

If you are the person who ends up holding the authority, the Consumer Financial Protection Bureau publishes free [Managing Someone Else's Money](#) guides, with separate booklets for agents under a power of attorney, court-appointed guardians, trustees and government fiduciaries. Its page was last modified in June 2026, and the Bureau states the guides are general consumer information rather than legal advice. Read the one that matches your role. Then keep records as if someone will check, because that is the job you accepted.

Because capacity and document rules vary by state, get prompt state-specific legal advice when you are unsure what document or authority is needed.

Tonight, write your parent's name at the top of a page and put one line under it: the last legal document you know of, and where you

think the signed original is. If the honest answer is that you don't know, that sentence is your first call tomorrow.

Chapter 17: Paying for Care

The message from the nursing home was still open on her phone when the daughter called her brother. They had assumed Medicare would pay for their mother's stay. He heard the pause after she said the word "assumed." Neither of them had asked what kind of care the bill described, or which part Medicare might cover.

This is an imagined family, not a prediction about yours. The answer is not simply "Medicare pays" or "Medicare pays nothing." It depends on the service, why it's needed, the provider, the coverage arrangement and, for some programs, where your parent lives. Start by separating medical care from the daily help that dementia can require.

This month's job is to get a written answer for one care need at a time. Ask the treating clinician what care is being recommended, then verify coverage with Medicare, the health plan or the program provider before relying on it. A diagnosis alone does not make every useful service a covered benefit.

The card is not a long-term care plan

Medicare.gov's [Long-term care page](#), checked in October 2026, says Medicare does not pay for most nonmedical long-term care. That includes ongoing help with daily living such as bathing, dressing and using the bathroom. The page also names home-delivered meals, adult day health care and transportation as examples of long-term care services that Medicare generally does not cover.

That sentence has sharp edges when you are already arranging the help. A parent may need someone there to cue lunch, stay nearby during a shower, or make sure the next trip is with a safe driver. Those needs can be real and costly without being skilled medical treatment. Medicare's long-term-care page says most health insurance and Medigap also do not pay for these services. Do not assume a service becomes covered just because a doctor recommends it.

There are covered medical services with conditions. Medicare's [home health services page](#) describes part-time or intermittent skilled nursing, certain therapies and limited home-health aide help when the person also qualifies for skilled services. Medicare says it does not pay for 24-hour home care, or for custodial personal care when that is the only care needed. Home health eligibility includes a clinician's order and other requirements, including being homebound. Ask the clinician and the agency to explain whether the actual service fits the current rules.

Short-term skilled nursing facility care is another distinct category. Medicare's [Skilled nursing facility care page](#) describes skilled nursing and therapy, not a general promise to pay for a nursing-home stay. Coverage has eligibility conditions. For Original Medicare, these include a qualifying inpatient hospital stay and a need for daily skilled care in a Medicare-certified facility; hospital observation time does not count as inpatient time. Medicare notes that some qualifying Accountable Care Organization arrangements may waive the three-day stay requirement. If your parent has Medicare Advantage, ask the plan and the facility, in writing where possible, whether the plan waives the three-day inpatient-stay requirement and what authorization, network, copayment, and facility rules apply. Ask the hospital's discharge planner and the facility what coverage route applies before signing a financial agreement.

I would not let the word “nursing home” answer the coverage question. It can describe a place offering several kinds of care, while Medicare’s rules turn on the service and the person’s eligibility. Chapter 19 is where we compare care settings. Here, write the service, the payer’s answer, any approval still needed, and the family’s estimated uncovered amount on one line.

There is a smaller benefit worth asking about if a clinician recommends it. Medicare.gov’s [Caregiver training services page](#), checked in October 2026, says Part B may cover training when the provider determines it belongs in the patient’s treatment plan and a caregiver’s help is needed for that treatment to succeed. The training can address tasks such as safe movement, daily care, communication or understanding the medical condition. It is not an open-ended payment for a home aide or a general class a family selects on its own.

The clinician identifies whether the training is appropriate and connects it to the treatment goals. The caregiver can receive an individual or group session without the patient present. With Original Medicare, Medicare.gov says the Part B deductible applies and then the caregiver pays 20% of the Medicare-approved amount. If your parent has a Medicare Advantage plan, ask that plan about its rules and costs. Ask the provider what skill the session will cover, whether it has determined the service fits the treatment plan, and what you may owe before the appointment. Do not treat “caregiver training is covered” as confirmation that every class or hour is covered.

Coverage can also be service-specific within one care plan. A home-health visit might include skilled nursing and a limited aide task during the same episode, while a separate daily personal-care shift remains outside Medicare’s home-health benefit. The Medicare [home health services page](#) says the aide service must be part-time or intermittent and delivered alongside qualifying skilled services. Ask

the agency to identify the covered portion and the separate service, rather than accepting one label for the whole arrangement. If the clinician's office, agency and plan give different answers, write down each person's name and ask the plan to resolve the coverage question before building a schedule around it.

A GUIDE participant is a provider, not a new Medicare benefit

The CMS Innovation Center's [GUIDE Model page](#) describes GUIDE — Guiding an Improved Dementia Experience — as a voluntary model with participating providers. CMS lists care navigation, a support line available around the clock, caregiver training and education, respite services up to an annual cap, and connections to community resources. The respite cap described by CMS is up to \$2,500 per patient per year, adjusted for inflation. That is a limit on a program service, not a check promised to every family or a reimbursement you should budget before a provider confirms eligibility and arrangements.

GUIDE is not a new category on the Medicare card. A provider must participate, serve the area and confirm that the person meets the model's current criteria. Ask the provider whether your parent is already aligned to another GUIDE participant and whether it serves the current ZIP code. CMS's FAQ, checked in October 2026, says the person must have confirmed dementia and Original Medicare Parts A and B as the primary payer. People enrolled in Medicare Advantage, PACE or the Medicare hospice benefit are not eligible. The person must live in a private residence or an eligible residential care community partnered with a GUIDE provider; long-term nursing-home residents and people living in a memory care unit are excluded.

CMS's FAQ also says residential care community residents may receive most GUIDE services if the setting has an approved partnership, but they are not eligible for GUIDE respite services. Rules and provider areas can change, so verify rather than relying on an old list.

The model is voluntary. A local provider must explain the services, get consent and submit information for CMS to confirm eligibility. CMS's current FAQ says aligned patients cannot be charged for GUIDE services, including respite. Ask the provider to say exactly which services are available to your parent, which local respite partners can serve them, whether their residence qualifies, and what is not included. A program description is not the same as an appointment or a care shift on your calendar.

Use CMS's [GUIDE participant list](#) to find providers serving your area, then call the provider rather than assuming the list means it has an opening or serves your parent's ZIP code. The diagnosing doctor's office may know a local team. The local Area Agency on Aging can point you toward other community services, even if GUIDE is not available.

Hospice has a different purpose

Hospice is not a general payment route for long-term dementia care. The Medicare.gov [Hospice care page](#) was current when checked in October 2026. To qualify under Part A, the hospice doctor and regular doctor, if there is one, must certify a terminal illness with a life expectancy of six months or less. The person must choose comfort-focused care instead of treatment intended to cure that terminal illness and sign the hospice election statement. Only the doctors make that certification. A family should not try to calculate eligibility from a diagnosis or a difficult month.

Hospice is care organized around comfort and support under a plan made with the hospice team. Medicare says covered hospice care can be provided at home or another place where the person lives. The team arranges the care included in that plan; it is not automatically a paid person sitting in the home all day. Ask what visits and help your parent's plan includes, who answers calls outside scheduled visits, how medicines and equipment are handled, and what to do if symptoms change overnight. Those are questions for the actual hospice provider, not assumptions based on a brochure.

The room-and-board distinction matters. Medicare does not cover room and board at home, in a nursing home, or in a hospice inpatient facility; a short-term inpatient or respite stay is covered only when the hospice team determines it is needed and arranges it. Medicare.gov says the person may owe 5% of the Medicare-approved amount for that respite stay. The hospice page also lists an outpatient prescription copayment of up to \$5 for each drug for pain or symptom management. The current page says there is no charge for covered hospice services from a Medicare-approved provider, but those stated costs and other coverage can apply. Confirm the exact plan and any amount due with the hospice and Medicare before enrollment.

Choosing hospice is a significant medical decision, not a shortcut to a room or a promise of constant supervision. If someone raises it, ask the treating doctor to explain the certification, what care would continue, what treatment choices change, and how the family can revisit the decision. Medicare.gov explains that a person can stop hospice care. A hospice provider should answer practical and coverage questions in plain language; if the answer is unclear, ask again before signing.

A coverage page you can use on the phone

A billing conversation gets easier when you are asking about one specific service. Keep Chapter 11's current health summary nearby, and write down the person's name, date, service, and answer. Ask the doctor's office what is medically recommended; ask the payer or provider what it will cover. One answer does not substitute for the other.

Use these prompts with the clinician's office, Medicare.gov, the plan, a home-health agency or a GUIDE/hospice provider:

- What is the exact service being recommended, and who will provide it?
- Is this skilled medical care, intermittent home health, short-term skilled nursing, hospice, GUIDE support, or ongoing help with daily living?
- What eligibility conditions, clinician order, prior approval or provider status apply to this person's situation?
- Does the person's Medicare arrangement qualify, and is the provider in the plan or program?
- What will Medicare or the program pay, and what could we owe for this service?
- Is any cost estimate written, and does it include supplies, transportation, room and board or extra hours?
- If coverage is denied or limited, who explains the reason and the next step?
- For GUIDE, does this participant serve this ZIP code and residence type, is my parent already aligned to another participant, and how would respite be arranged if eligible?
- For hospice, what support is in the written plan, who answers after hours, and what costs are excluded?

Keep the answers separate. “The doctor ordered it” means the clinician recommended it; it does not establish that Medicare will pay. “The plan covers home health” doesn’t establish that it will pay for daily personal care without skilled services. “A GUIDE participant is in our state” doesn’t confirm the provider covers this ZIP code or that your parent qualifies. Write down the exact service and the person who verified each answer.

For nonmedical help, ask the local Area Agency on Aging what services exist and what their eligibility, cost, waiting list and hours are. CMS’s GUIDE participant list and the Eldercare Locator are starting points, not promises of a provider or available shift. Medicare.gov explains some services Medicare does not cover; a state Medicaid agency, veterans’ benefits office or long-term-care insurer can explain any separate program that might apply. Rules for these programs differ, and a professional familiar with the family’s finances or benefits may be needed before making a decision. Do not transfer assets or make a financial move based on this chapter.

For the coverage work, use one line per service rather than one answer for the whole care plan. If the estimate covers three weekly aide visits, that is not the same as the daily schedule your parent needs. Write the gap beside the estimate and assign someone to ask about that gap. Do not fill it with an assumption that Medicare, GUIDE or hospice will cover it.

Tonight, choose one service your parent needs now. Write down its name, who recommended it, and the first office you will call to verify coverage. Leave a blank line for the answer and the next person who owns it.

Chapter 18: Respite and Your Own Health

Maggie didn't plan to end up in urgent care with a band of blisters wrapping her ribs. Shingles doesn't ask permission. It showed up in the spring of 2020, and for ten days she could barely tolerate a shirt against her skin, let alone help her mother to the bathroom.

That week, Dot went to stay with Earl's sister two towns over as a hurried stopgap, not an established respite arrangement. Earl had been doing daily care, with Maggie taking on more of it. Maggie had been meaning to look into adult day programs for months. She'd made one phone call the previous fall, written the number on a sticky note, and let the sticky note slide behind the toaster. The shingles did the deciding for her.

When she recovered, she enrolled Dot in adult day two days a week, not because the disease had gotten worse, but because Maggie had learned, the hard way, that her own body was also part of the care plan. She still remembers how strange it felt the first Tuesday, standing in her own kitchen at ten in the morning with nothing urgent in front of her, unsure what to do with an hour that wasn't already claimed.

Finding help that fits

Respite means someone else takes the caregiving for a stretch of time, planned in advance, so you can rest, work, see a doctor, or just sit somewhere quiet.

It's also not a single arrangement you make once and never revisit. What works for your parent at diagnosis may stop working eighteen months later; the daily-care needs from Chapter 15a and the behavior patterns from Chapter 15 can change what kind of respite actually helps. Expect to reassess this every few months rather than treating one program as a permanent fix.

A few places to start asking:

Adult day programs. These run during daytime hours, often with structured activities, meals and supervision suited to people living with dementia. Ask about staff-to-participant ratios, dementia-specific training, transportation, and whether the schedule can flex if your parent has a hard morning. Two days a week was Maggie's arrangement, not a recommendation; ask the program and your parent's care team what schedule and supervision fit your parent's needs.

In-home respite. A trained aide or volunteer comes to the house so you can leave, or so you can be present but off duty for a few hours. Ask who does the hiring and background checks, what tasks the aide can and cannot do, and what happens if your parent refuses the visitor that day.

Short facility stays. Some assisted living communities and nursing homes offer short overnight or multi-day respite stays, useful for a family event, a procedure of your own, or simply a week of real sleep. Ask about cost, what medical information the facility needs beforehand, and how your parent will be oriented to an unfamiliar place. A short stay is also a quiet way to see how a facility actually runs, which matters later if Chapter 19's question of memory care ever comes up; you're not committing to anything by asking.

Family and friends, used deliberately. A sibling who lives two hours away, a neighbor who has offered "anything you need," a church or synagogue's visitation list — these are real respite resources, not consolation prizes next to professional care. The

difference is planning instead of improvising. Ask for a specific block of time, give clear instructions in writing, and treat a willing relative's help the same way you'd treat a paid aide's: with a plan for what to do if something goes wrong while you're away.

The GUIDE Model's respite funding. Chapter 17 described the CMS GUIDE Model's yearly respite benefit, available only through a participating local provider once eligibility is confirmed. If a GUIDE provider already serves your area, ask specifically how its respite funding is arranged and what it covers; don't assume every GUIDE participant offers the same respite partners or hours.

These routes are not interchangeable, and none of them comes with a promise of being available, affordable or nearby when you call. The [Eldercare Locator](#) is a free public service of the Administration for Community Living. Search the Eldercare Locator online or call/text 1-800-677-1116 for help connecting with local services; confirm openings, costs and eligibility with providers. Its website, checked in October 2026, also offers chat and email access to trained staff.

You may need more than one backup: a predictable adult-day slot and a separately agreed emergency substitute serve different needs. Build the combination around your actual week, not around whichever option sounds the most official. A willing helper still needs the skills and support to meet your parent's current needs; an offer alone doesn't establish that.

A shift someone has actually accepted

Try a suitable provider or helper while you can stay for an introduction and check the arrangement, rather than making the first call during an emergency. The Alzheimer's Association's [Respite Care guidance](#), checked in October 2026, recommends non-emergency trials and accessible medical-team contacts and current

medication information. That is the prepare-early lesson. The next job is making the handoff usable.

Here is a made-up example, not another incident from Maggie's family. A daughter needs coverage for her own Thursday medical appointment. Her brother has offered to help. They agree on 9:30 a.m. through 12:30 p.m., including travel time, at their mother's house. He replies in writing, "I can cover Thursday, 9:30 to 12:30." Both put it on their calendars. "I'll try to get over there" would leave the shift unfilled.

Before accepting, he has spent time there with his sister present, gone over their mother's current care instructions, and confirmed he can provide the supervision and daily help needed during that window. Their mother's mobility help has already been reviewed with the care team; he has received the instruction needed for his part. If a task is beyond him, the family asks for qualified help rather than improvising a transfer. This example assumes that suitability check is complete, not that being a brother makes him qualified.

The daughter seeks her mother's agreement where possible and shares the necessary care information privately with the agreed helper, using Chapter 11's handoff boundary. He receives the current instructions for supervision, mobility, meals and personal care, with any prescribed eating or swallowing precautions. The medication list is dated and includes the amounts and times actually prescribed. They identify who is responsible for any medicine due during the shift and confirm that person can carry out the instructions. Unclear directions go to the prescriber or pharmacist, not to a guess between siblings.

They walk through where the care instructions and medicines are kept. He has the clinician's office and after-hours numbers, the daughter's number, and the emergency plan. They check that he can find the information without borrowing her phone. She doesn't hand

him a folder at the door and assume that reading it later will be enough.

In this example, an appropriate home-care agency has separately confirmed an alternate worker for that window if the brother cancels. The daughter has the agency's contact and the cancellation instructions in writing. The fallback is an actual agreement, not a number on a list. If neither helper can attend, she contacts her clinician's office about the appointment and seeks another safe arrangement. She does not leave her mother alone while hoping someone turns up.

On Thursday, the brother arrives before she leaves. They review any change since the written plan, and he confirms he is taking over. She gives the appointment office as an alternate contact if she cannot answer during the visit. If an emergency occurs, he follows the emergency plan and calls 911 for immediate danger rather than waiting for her reply. If she is delayed, she contacts him before the agreed end time and confirms continued coverage or another suitable handoff.

When she returns, he reports what happened, including any care difficulty or call he made. They update the instructions if needed. The daughter has attended her appointment, and her mother has had an identified caregiver throughout. That is what the confirmed shift accomplished; it doesn't prove the same arrangement will fit a later change in needs.

Watching your own thresholds, not just your own tiredness

Caregivers are told constantly to "take care of themselves," which is true and nearly useless as advice, because it doesn't say when or how. Here are thresholds, not vague reminders.

If sleep, eating, mood or daily functioning is affected, contact your own clinician; don't wait two weeks if symptoms are severe, worsening or interfering with safe care. If care cannot be provided safely today, get urgent help now. The National Institute of Mental Health's [My Mental Health: Do I Need Help?](#), revised in 2025 and checked in October 2026, directs provider contact when symptoms worsen. Its discussion of two weeks is not a requirement to delay help.

Seek help urgently if you reach a point where you can no longer provide safe care — yours or your parent's. That might mean you're too exhausted to catch a fall, too overwhelmed to remember medications, or so depleted that ordinary frustration starts to frighten you. That is not a personal failure. It is information that the current arrangement has to change, today, not after one more hard week.

If anyone is in immediate physical danger, call 911. For thoughts of self-harm or an emotional crisis, call or text 988; if that crisis is immediately life-threatening, call 911. NIMH's [Help for Mental Illnesses](#), reviewed in April 2026, distinguishes emergency help from crisis counseling. The [988 Suicide & Crisis Lifeline](#) is free, confidential and available by call or text, 24 hours a day, every day. Its counselors support people in suicidal crisis or emotional distress; you don't need to wait for a life-threatening moment to contact them.

The Alzheimer's Association's [24/7 Helpline](#), at 800.272.3900, is a separate, free resource built specifically for people living with dementia, their caregivers and families. It's reachable any day, any time, staffed by real people. They talk through crisis assistance, emotional support and local referrals, including, when relevant, the respite options this chapter is about. Call it when you need someone who already understands the territory, not just a sympathetic ear.

I keep both numbers on the inside of a kitchen cabinet, not buried in a phone I might not reach in a bad moment. Write them somewhere visible before you think you'll need them.

Tell your doctor you are the primary caregiver, then describe the sleep problem or headache in your own words so they can assess it.

Keeping your own care from sliding

It is easy to let your own checkups slide while everyone's attention, including yours, goes to your parent's appointments. The blood pressure medication you meant to refill. The dentist visit you canceled twice. None of it feels urgent in the moment a parent needs help right now.

Treat your own medical care the way Chapter 11 asked you to treat your parent's: write it down, schedule it, and don't let "later" become the plan. If you can't get to your own appointment without coverage, that's a respite question, not a reason to skip the appointment. Ask the adult day program or an in-home aide to cover exactly that window.

Watch, too, for the quieter signs that don't look like a crisis: skipping meals because cooking feels like too much, losing track of your own medications while managing your parent's, snapping at people you love and then feeling ashamed of it. These are not character flaws. They're signals that the respite plan, or the lack of one, needs attention.

None of this is an argument for perfection. Some weeks the respite plan holds and some weeks it doesn't. A missed adult day session, because your parent refused to get in the car that morning, is not evidence the whole arrangement failed. The point is to have a plan worth returning to, not a plan you abandon the first time it's inconvenient.

Set the guilt down, and call the program back to reschedule.

What to ask before you commit to a program

A short list to bring to any respite conversation, adult day, in-home or facility-based:

- What are the hours, the cost, and is there a waiting list?
- What is the staff-to-participant or staff-to-client ratio, and what dementia-specific training do staff have?
- What happens if my parent refuses to go, or becomes agitated while there?
- Who do I call if something changes during the day, and how quickly will they reach me?
- Is transportation included, and what if my parent needs help getting in and out of a vehicle?
- If this is a GUIDE participant's respite service, is my parent already confirmed eligible, and what exactly does the funding cover?
- Can you safely manage my parent's current supervision, mobility and daily-care needs? Who covers a cancellation? Who has the current care instructions, medication list, clinician contacts and emergency plan?

Write the answers down. A program's reputation, or a friend's good experience with a different parent, is not the same as its answer to your specific questions about your own parent's needs.

When to get help for yourself

Keep this card where you can find it. The crisis numbers and their different purposes were checked against NIMH, the Lifeline and the Association's official sites in October 2026.

Immediate physical danger: 911. Self-harm thoughts/emotional crisis: 988 (call/text). Cannot provide safe care: seek urgent help and

arrange safe coverage now. Caregiver-specific help: Alzheimer's Association 800.272.3900. Sleep, eating, mood or functioning affected: contact your clinician; sooner for severe/worsening symptoms.

If an emotional crisis is immediately life-threatening, call 911. The Association Helpline offers caregiver support and referrals, not emergency dispatch. If you cannot provide safe care today, contact the care team or an urgent local care resource now for help arranging suitable coverage; call 911 if anyone is in immediate physical danger. Don't leave your parent without needed supervision while arranging help.

Tonight, write the name of the person or provider who has actually accepted the next shift, and confirm the handoff. Contact your clinician about changes to your own sleep, eating, mood or ability to function, without a two-week waiting rule. Unsafe care today needs urgent help today; immediate physical danger means calling 911.

Chapter 19: When to Consider Memory Care

The appointment is ending. Your coat is on, but the question you came with is still sitting in your notebook. Imagine asking the care team: “Which needs can we safely meet at home now, and which need a different setting?” That’s a hypothetical caregiver question, not a remembered exchange from Maggie’s family. It asks for something more useful than a date: an assessment of the care your parent needs and the help actually available.

The signs worth naming out loud

In-home care reaches its limit in a few recognizable ways, not one dramatic event.

The first is safety. A fall that could have been worse. A near-miss out a door, the kind Chapter 13 walked through. A stove left on more than once despite the measures you put in place.

If your parent is missing, in immediate danger, or has sudden confusion or possible stroke signs, call 911 now. For a fall or missed medicine, contact the treating clinician or pharmacist promptly for individual advice; do not guess at a make-up dose. Once urgent needs are addressed, bring the pattern of near-misses to the care team to reassess supervision.

Use the emergency boundaries from Chapters 10, 11 and 13, including emergency help for injury or inability to get up after a fall. The CDC’s [2026 stroke guidance](#) directs immediate 911 for possible stroke signs; the NIA’s [falls guidance](#) says to get help if hurt or

unable to get up and report falls to the doctor. A facility search never takes priority over help needed now.

The second is your own health and capacity, the ground Chapter 18 covered in detail. If respite isn't closing the gap anymore — if you're exhausted on the good weeks, not just the hard ones — that's not a personal failing to push through. It's information.

The third is a mismatch between what your parent now needs and what can realistically happen in a house, even with help. Overnight supervision that no single aide schedule covers. Medical needs that call for more hands, or more trained hands, than you can line up. Behavior changes — discussed in Chapter 15 — that have moved past what tracking and small environmental fixes can manage, where the care team is recommending a different setting, not just a different approach.

None of these signs, alone or together, means you've failed at something. They mean the job has changed size. A house can only hold so much supervision before supervision itself becomes the whole day, for one or two people, instead of a staffed team working in shifts.

Sometimes the concern isn't visible to the relative who visits on Sunday. A missed dose wasn't noticed when it was due. An attempt to leave was caught only because a neighbor happened to be in the yard. Nobody slept because every sound from down the hall meant getting up to check. Tell the care team what happened and what supervision was available; don't wait for several incidents to justify asking for help.

After immediate needs are addressed, the dated observations from Chapter 11 can help explain the remaining care gap. Include what you don't know. An unwitnessed event and a confirmed one aren't interchangeable, and a quiet afternoon doesn't establish that overnight supervision is adequate. Ask what must change now while the longer-term options are being assessed.

What “memory care” actually means

The phrase gets used loosely, so it helps to know what it’s standing in for. According to the National Institute on Aging’s [Finding Long-Term Care guidance](#), checked in October 2026, facility-based long-term care includes assisted living, group homes and nursing homes. The differences matter more than the label on the brochure.

Assisted living facilities help with daily tasks — bathing, dressing, medications, meals — but don’t provide around-the-clock medical care. Some have a specific Alzheimer’s or dementia unit, often called “memory care,” with added staffing, structured activities and a secured layout designed around wandering risk. Not every assisted living facility has one, so don’t assume the general label covers dementia-specific support; ask directly.

Group homes are smaller, often a converted house, with a caregiver on-site at all times. A smaller setting may appeal to your parent; ask how it handles the care and medical support they actually need. The NIA notes that group homes may not be state-inspected the way other facility types are, which makes it worth asking directly how the home is licensed, who oversees it, and what happens if a problem arises.

Nursing homes provide ongoing medical care and are state-inspected and regulated; some have a separate dementia-specific unit. These suit people whose medical needs go beyond what assisted living or a group home can provide, not just dementia symptoms on their own.

None of these is automatically “better.” The right fit depends on your parent’s current medical needs, the behaviors involved, your budget, and what’s actually available near you or near whichever family member can visit regularly.

What to ask on a tour

Before selecting a facility, ask the care team which level of care to tour. NIA's [facility-selection guidance](#) recommends asking a health care provider about needed services and considering the person's preferences. Bring this list, and don't let a glossy lobby answer questions it wasn't asked.

- What's the staff-to-resident ratio, during the day and overnight?
- Can your staff safely meet my parent's current supervision, mobility, eating and medication needs on evenings and weekends; what happens if those needs change suddenly, and who decides when this placement is no longer suitable?
- What dementia-specific training do direct-care staff receive, and how often is it refreshed?
- What does a typical day look like — structured activities, free time, outdoor access?
- Is there secured or supervised outdoor space, not just an indoor common room?
- How does the facility handle agitation, wandering attempts, or a resident who refuses care that day?
- What's included in the base fee, and what costs extra — incontinence supplies, medication management, transportation to appointments?
- How is the facility licensed and inspected, and can you see the most recent inspection report?
- What's the process if your parent's needs increase — does the facility handle that internally, or would another move be required?
- How are families kept informed of day-to-day changes, falls, or medical concerns?

- Can you visit unannounced, and can you join a meal or activity before deciding?

Bring the current care instructions, shared appropriately, and ask the person responsible for care to explain how staff would carry them out. If your parent has prescribed swallowing precautions, ask who can follow them at every meal. If transfers require trained assistance, ask who is available to provide it. Ask who administers medicine, who can contact a clinician overnight, and how an urgent change is escalated. For exit precautions, ask how supervision and safe emergency egress work together, using Chapter 13's boundary rather than treating a secured door as the whole answer.

After each visit, date the notes and record the unanswered questions. This week, contact one search resource.

How payment questions fit in

Ask admissions for an itemized written price and which payers it accepts. Verify eligibility and what each service is covered for with Medicare or the health plan, your state Medicaid agency, or the long-term-care insurer before relying on an estimate; get professional advice before signing an unclear contract.

As Chapter 17 explained, Medicare's [long-term-care coverage page](#) distinguishes nonmedical long-term care, which Medicare does not pay for, from potentially covered services with separate requirements. A facility's willingness to accept a payer does not establish your parent's eligibility. State Medicaid rules vary; check with your parent's state agency about this setting and these services, not just the word "care."

If GUIDE currently helps your family, tell the participating provider the precise proposed setting before relying on any continued services. CMS excludes memory-care-unit and long-term nursing-home residents; even eligible residents of partnered

residential care communities cannot receive GUIDE respite. Ask what will stop and what other support is available.

The [CMS GUIDE FAQ](#), revised September 25, 2026, states that memory-care-unit residents are ineligible as of July 2026. Approved residential-community partnerships are required beginning July 1, 2026; eligible residents may receive other GUIDE services, but not respite. Have the participating provider assess the exact placement rather than guessing from an assisted-living brochure.

Where to start looking

A few resources, named in Chapters 13 and 18 for other purposes, do double duty here. The ACL's [Eldercare Locator](#), or 1-800-677-1116, connects families to local aging services. Ask about the local Area Agency on Aging and help finding appropriate facilities. NIA directs readers to [LongTermCare.gov at ACL](#) for housing options and local-service information. If that page won't load, use NIA's linked facility guidance above or contact the Locator instead.

Medicare's [Care Compare](#) lets you look up nursing homes, including inspection results and staffing data; it isn't a directory of every assisted living facility or group home. Licensing and inspection for those settings vary by state. Ask your state's licensing agency how to check the named facility's current license and inspection history. Medicare's [staffing explanation](#) cautions that its averages don't show staff present at every particular moment or care given to one resident. Use the figures to ask questions, not to certify an evening shift you haven't seen.

Ask prospective facilities about current openings and admission requirements before scheduling a visit. A place on a resource list is a lead, not confirmation that it can accept your parent or provide the needed care.

A tour that went better than expected

Here's a made-up visit, not a Maggie-and-Dot story: a worried daughter and son tour a facility together, expecting a hard afternoon. They'd pictured something closer to a hospital hallway. Instead, a staff member kneels down to talk with a resident who's anxious about finding her room, uses her name, and waits for an answer before moving on. The daughter notices this before she notices the furniture. Later, over coffee in the car, the siblings admit the tour reassured them more than either expected, though they're careful to say one good interaction isn't the whole picture. They schedule a second visit at dinnertime to see another shift and ask how many trained staff are actually on duty then.

Carry that hypothetical visit through the less reassuring part: comparing answers. The siblings have permission to share their mother's current instructions. In this invented example, she needs supervision overnight, trained help with transfers, assistance following a prescribed swallowing plan at meals, and administered medicines. Those are her assessed needs in the example, not a list that applies to everyone with dementia. Her clinician has asked them to investigate settings able to meet those needs; the assessment isn't a promise that any particular place can.

The first facility's admissions representative says, "We have staff here all night." The daughter writes that answer under overnight supervision but leaves the question open. It doesn't say who can respond when her mother gets up or how staff cover several residents needing help together. She asks to speak with the care director. She wants an explanation of this mother's supervision, not another description of the building's opening hours.

At the second visit, that director explains how overnight help is assigned and identifies the trained workers available on the shift being discussed. The director says the facility would review the mother's transfer instructions before deciding whether it could

accept her. The siblings record that as an assessment still needed. They do not turn “we’ll review it” into “they can do it,” or assume today’s staff count will hold every future night.

Another facility gives a different answer: it cannot provide the assistance described in the current transfer instructions. The son likes its smaller dining room, but writes “current transfer needs not met” beside its name. That isn’t a poor score to average against the nice room. It is a specific mismatch to take back to the care team. This example doesn’t establish that all small facilities have the same limitation.

For eating, the first facility names the person who would review the prescribed swallowing plan and asks for the current instructions before confirming suitability. The siblings ask who would provide the required assistance during weekend meals. No answer yet. Under medicine administration, they record which staff the director says would administer the medicines, then ask whether that service is included in the written price. They aren’t asking staff to change the prescription or interpreting swallowing instructions themselves.

They send the dated answers and open questions to the clinician and social worker through an agreed private channel. Their question is narrow: does either setting’s described care meet the mother’s assessed needs, and what must be confirmed before choosing? The social worker asks them to obtain the first facility’s individualized care assessment and written account of what happens if needs increase. The clinician asks for clarification of the meal assistance. Neither professional has approved placement merely because a tour occurred.

One payer question is still unresolved. The long-term-care insurer hasn’t confirmed whether the mother’s policy covers this particular facility and the quoted services. The daughter labels that amount unconfirmed rather than subtracting it from the price. If GUIDE is helping now, she also asks the participating provider about the

proposed memory-care unit; the CMS exclusion above cannot be treated as a benefit that continues until someone sends a bill.

The siblings have not chosen a facility by the end of the example. They have ruled out an identified mismatch and know which care and payment answers are missing. The care team helps them address current supervision while those questions are resolved. An unfinished comparison is useful when it tells you whom to call; it becomes dangerous when “still looking” is used to excuse unsafe care today.

When no option feels good.

Grief can come with you; write your parent’s specific needs, each facility’s answer, and one question for the clinician or social worker. Keep preferences beside care needs: your parent may care deeply about an outdoor place to sit or being close enough for visits. Record those wishes without letting them erase a necessary care requirement.

Use a separate line for each need, with the visit date and the name or role of whoever answered. Mark “not confirmed” where the answer depends on an assessment or a promised callback. Ask which unanswered issue prevents a safe choice; don’t give every blank the same weight. Carry the most consequential question into the next professional conversation. This comparison page is a practical aid, not a clinical score or permission to override your parent’s decision.

For the belongings, house and moving-day logistics, see *Downsizing a Parent*.

The conversation with your parent, if they can still have it

Depending on where your parent is in the disease, this decision may happen with them, around them, or largely without their input — and it helps to know which situation you’re actually in before you start talking. If your parent can still follow and weigh in on the conversation, include them honestly rather than presenting a move as already settled. Frame it around something concrete they care about: more help during the day, people nearby instead of alone for long stretches, a setting that can catch a fall faster than you can from another room. Avoid the word “home” as a trap — telling someone a facility is “just like home” when it plainly isn’t erodes trust fast, and dementia doesn’t erase the ability to notice a broken promise.

If your parent cannot make this particular decision, ask the care team about their decision-making ability and consult the valid documents and a licensed elder law attorney familiar with your parent’s state to establish who may decide and sign. Keep your parent involved in the ways they can participate; do not assume that being the lead caregiver grants authority.

That is the Chapter 16 authority boundary, not a new family test of capacity. The Alzheimer’s Association’s [Legal Documents guidance](#) says decision rights remain while the person has legal capacity and laws vary by state. Ask the attorney about authority for this decision and these admission papers. Include appropriately involved relatives before a decision becomes urgent, while keeping family consultation separate from the legal right to decide.

Give yourself time you may not think you have

If you can see a care gap developing, begin asking about appropriate settings before a discharge deadline forces several questions into one

afternoon. You cannot forecast every change, and starting early doesn't establish that an opening will be available.

Ask the discharge planner and care team whether an appropriate short-term setting is available and safe for your parent's present needs; confirm its admission requirements, cost and who provides care afterward. Do not leave an unsafe discharge or supervision gap while waiting.

Use NIA's linked long-term-care guidance to support that conversation, not as proof that a short stay exists nearby. Tell the discharge team plainly what care cannot be provided at home. Leave the next conversation with the name of the person following up on that gap, not just a brochure on the passenger seat.

A Little More Room to Live

With Mama's hymn over tea still in my ears, I want to leave you without pretending the work is finished.

Mama's opinions about music were hers; my reading of an expression was not a decision.

You may have arrived at this page with several matters still open. Reading to the end hasn't closed them. The work you've already done stands beside those unanswered questions.

The title says first year because a book needs a boundary. A family doesn't receive one with the diagnosis. The worry may have started earlier; the name for it may arrive later. A month can hold both familiar habits and a new care question, sometimes before anybody has found words for the change. The calendar keeps its own pace regardless. I use livable for the part that can't be measured by a date or a finished answer. It doesn't promise that a difficult thing will become easy, or that the next year will follow this one. It is a modest word for a stretch of time that still contains ordinary days.

You cleared a walking route and checked the light by the chair. Some evenings still went badly. Nobody else may have seen the work you did before they arrived.

The current-care boundary in Chapter 19 still stands: help needed now comes before an ordinary planning task or another page of reading.

Chapter 18's safe-coverage condition still matters. Once someone suitable has taken the shift, the hour belongs to you. You may sit through it without coming back with a better plan.

For the ordinary planning work, choose one unfinished item from a checklist you already used in this book. One actual task that still matters to your family. Give it an owner who has accepted it and a drop date, and put that agreement beside the item. The task may be yours. It needn't be.

Leave the book open at that line.